UNITED STATES DISTRICT COURT

SOUTHERN DISTRICT OF TEXAS

GALVESTON DIVISION

JERI PEARSON, ELIZABETH KLEM, BEN HOMAN, and ROB FOWLER	*	
Plaintiffs,	*	
VERSUS	*	CIVIL ACTION 3:23-cv-387
SHRINERS HOSPITALS FOR CHILDREN SHRINERS HOSPITALS FOR CHILDREN -- TEXAS, BEVERLY BOKOVITZ, FRANCES FARLEY, JERRY GANTT, JOHN McCABE, PHILLIP GRADY, AND CECILE ERWIN YOUNG	*	
Defendants	*	

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AMENDED COMPLAINT
(JURY TRIAL REQUESTED)

NOW INTO COURT, through undersigned counsel, come Plaintiffs Jeri Pearson, Elizabeth Klem, Ben Homan, and Rob Fowler ("Plaintiffs"), who file this Amended Complaint against Defendants, Shriners Hospitals for Children, Inc., Shriners Hospital for Children – Texas, Beverly Bokovitz, Frances Farley, MD, Jerry Gantt, John McCabe, Phillip Grady, and Cecile Erwin Young ("Defendants"), presenting allegations and causes of action as follows:

DESCRIPTION OF CAUSE OF ACTION

This lawsuit is brought under 42 U.S.C. §1983 seeking damages for deprivation of rights granted to Plaintiffs by the United States Constitution, 21 U.S.C. §360bbb-3, *et seq* ("EUA statute"), 42 USC §247d-6d, *et seq* ("PREP Act"), 45 CFR Part 46, the common laws of the State of Texas, and fundamental rights as a human being to refuse administration of a COVID-19

EUA/PREP Act investigational drug, and to hold accountable Shriners Hospitals for Children and Shriners Hospital for Children – Texas, State Actors at all times pertinent herein, and its PolicyMakers, the Chief Nursing Officer (CNO) Beverly Bokovitz, Chief Medical Officer Frances Farley, MD, Jerry Gantt, John McCabe, and Phillip Grady (Shriners and its named executives will be collectively referred to as "Shriners PolicyMakers") and Cecile Erwin Young, in her individual capacity. The FDA defines the drugs at issue as "investigational with no license for any indication" and "legally distinct" from the FDA-licensed COMIRNATY®. Even though Defendants' mandate was instituted during and in response to a pandemic emergency, as the U.S. Supreme Court noted: **"even in a pandemic, the Constitution cannot be put away and forgotten."** *Roman Catholic Diocese of Brooklyn v. Cuomo*, 141 S.Ct. 63, 208 L.Ed.2d 206 (2020).

I. INTRODUCTION

1. The federal government's executive branch (the Department of Defense) purchased all COVID-19 drugs authorized for emergency use and introduced them into commerce through the federally funded CDC COVID-19 Vaccination Program ("CDC Program").

2. The federal government then recruited states to help distribute and administer the unlicensed drugs to the American people using the States' existing Immunization Information Systems (IIS).

3. The drugs in question were "investigational" and "not licensed for any indication"[1]

4. The terms "investigational" and "investigational new drugs" are deemed to be synonymous per 21 CFR 312.3.

5. An investigational new drug does not have a legal indication to treat, cure, or prevent a disease and therefore is not considered a vaccine.

[1] See Exhibit B, August 23, 2021 EUA Letter from FDA to Pfizer.

6. As an example, the Pfizer-BioNTech COVID-19 Vaccine drug does not have an FDA-approved legal indication to prevent the Coronavirus.

7. A drug is approved or investigational according to the product's labeling.

8. Once a drug is labeled "investigational," it can never be considered FDA-licensed, even if the formulation received approval because the investigational drug was not manufactured, distributed, or administered under the same process or laws applicable to the Biologics License Application (BLA) process[2], which is the only way a drug obtains FDA licensure.

9. The Pfizer-BioNTech COVID-19 Vaccine, Moderna Vaccine, and Janssen Vaccine are investigational drugs authorized under 21 U.S.C. §360bbb-3 ("the EUA statute") and listed as countermeasures under 42 U.S.C. §§ 247d-6d,6e ("the PREP Act").[3],[4]

10. Thus, the drugs are EUA/PREP Act investigational drugs.

11. EUA drugs are manufactured in facilities that are not BLA-approved.

12. No person has the authority to place another person under threat of penalty for refusing the administration of a drug, medical countermeasure, or activity authorized under the EUA statute or the PREP Act.

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[2] 21 U.S.C. § 355

[3] The terms "investigational new drug" and "investigational drug" are deemed synonymous by 21 CFR 312.3. Investigational new drug "means a new drug or biological drug that is used in a clinical investigation." (21 CFR 312.3 "Investigational new drug") Clinical investigation "means any experiment in which a drug is administered or dispensed to, or used involving, one or more human subjects. For the purposes of this part, an experiment is any use of a drug except for the use of a marketed drug in the course of medical practice." (21 CFR 312.3 "Clinical investigation"). Also, investigational new drug means, "A substance that has been tested in the laboratory and has been approved by the U.S. Food and Drug Administration (FDA) for testing in people…" NCI Dictionary of Cancer Terms. National Cancer Institute. Published 2023. Accessed June 25, 2023. https://www.cancer.gov/publications/dictionaries/cancer-terms/def/investigational-new-drug

[4] In *Johnson v. Brown*, 567 F.Supp.3d 1230 (D. Or. 2021), the court, considering whether or not the drugs were "experimental" for the reason of *jus cogens,* erred in stating, "Even if the terms "experimental" and "investigational" are interchangeable, which they do not appear to be." Investigational was an adopted term because it is more pleasing terminology than experimental, but the words are synonymous for legal purposes. Not every use of an investigational new drug is considered an "experiment" under a variety of conditions established by HHS, NIH, CDC, and FDA. However, every use of an investigational drug is under a research activity, including those required under each EUA requiring the oversight of an Institutional Review Board for the protection of humans involved in federally funded investigational products. Plaintiffs do not allege that the CDC Program was an experiment, only that the Program required research activities.

13. The State of Texas volunteered to engage in the joint conduct of implementing the federal government's CDC Program, causing the State of Texas ("State") to owe constitutional obligations to citizens subjected to the CDC Program.

14. The CDC COVID-19 Vaccination Program Provider Agreement ("Provider Agreement") sets forth the legal requirements for protecting humans who are offered investigational drugs.

15. States that enter into the Provider Agreement must abide by its terms.

16. Signatory states must ensure that healthcare providers who sign the Provider Agreement abide by its terms in addition to State Immunization Information Systems (IIS) requirements.

17. On September 14, 2021, after entering into the Provider Agreement, Shriners' "Joint Boards" approved a COVID-19 vaccine policy ("Shriners Policy") that threatened to penalize any "employee," "volunteer," and visitor to Shriners' facilities if they refuse to become "fully vaccinated against COVID-19" and have their "first vaccination shot by October 11, 2021."

18. The Shriners Policy was unlawful and unconstitutional because it exclusively relied on federally funded investigational drugs for compliance, drugs that can never be mandated with penalties for non-compliance.

19. Drugs and devices authorized under the EUA statute and the PREP Act and federally funded require the "legally effective informed consent" [5] of individuals before administration.

20. "Legally effective informed consent" requires the person sponsoring the product (i.e., the federal government and the State of Texas) or acting on behalf of the sponsor (Shriners

[5] 45 CFR 46.116(a)(1)

Hospitals for Children) to establish a legally approved environment ensuring the individual considering participation is not under "sanctions," "coercion," "undue influence," or "unjustifiable pressures" to use the product.

21. Defendants are legally sophisticated in the classification of drugs and with the EUA statute.

22. Defendants knew or should have known that no person can come under outside pressures to be injected with an EUA/PREP Act drug because the drugs are unlicensed.

23. Such outside pressures usurp from Congress its authority to prohibit the mandatory use of EUA/PREP Act drugs.

24. Defendant, Cecile Young, had a ministerial duty to execute the State's duties assumed by its voluntary participation in the CDC COVID-19 Vaccination Program.

25. Ms. Young's deliberate indifference to her official duties as Executive Commissioner of Texas Health and Human Services enabled Shriners PolicyMakers to violate their duties under the Provider Agreement and deprive Plaintiffs of their statutory and constitutional rights to be informed of the voluntary nature of a PREP Act program and to accept or refuse an EUA drug without threat of penalty, thereby directly causing Plaintiffs' financial, emotional, and legal injuries.

26. At all times pertinent herein, Defendants' actions were unlawful, *ultra vires*, and authorized only under a state-enforced custom of penalizing individuals who exercise their federal statutory right to refuse an EUA/PREP Act drug.

II. JURISDICTION AND VENUE

27. This Court has original jurisdiction under 28 U.S.C. §§ 1331 and 1343.

28. The civil rights portions of this action raise federal questions under the Spending Clause and the Fourteenth Amendment to the U.S. Constitution.

29. This Court has original jurisdiction under 42 U.S.C. §§ 1983 and 1988.

30. This Court has the authority to award costs and reasonable attorney's fees under 42 U.S.C. § 1988.

31. This court has supplemental jurisdiction over Plaintiffs' state law claims.

32. This Court has personal jurisdiction over Defendants as they are domiciled within this Court's jurisdictional boundaries.

33. This Court has subject matter jurisdiction over the parties because all acts complained of herein were committed by Defendants in the State of Texas and caused damage and/or deprivation to the Plaintiffs listed herein.

34. Venue is proper in this court because all events underlying the claims in this Complaint occurred in the State of Texas, which is situated within this Court's jurisdiction, and all Defendants reside in the State of Texas.

III. PLAINTIFFS

35. The following individuals are plaintiffs herein:

35.1. Plaintiff Jeri Pearson is an adult individual who, at all times pertinent, resided in the State of Texas and was previously an employee of Shriners in Texas.

35.2. Plaintiff Elizabeth Klem is an adult individual who, at all times pertinent, resided in the State of Texas and was previously an employee of Shriners in Texas.

35.3. Plaintiff Ben Homan is an adult individual who, at all times pertinent, resided in the State of Texas and was previously an employee of Shriners in Texas.

35.4. Plaintiff Rob Fowler is an adult individual who, at all times pertinent, resided in the State of Texas and was previously an employee of Shriners in Texas.

IV. DEFENDANTS

36. The following are named as defendants herein:

36.1. Defendant, Shriners Hospitals for Children, Inc., is a charitable 501C (3) non-profit corporation incorporated in the State of Colorado and headquartered in Tampa, Florida.

36.2. Defendant, Shriners Hospital for Children – Texas, at all times pertinent, was a subsidiary of Shriners Hospitals for Children, was the employer of Plaintiffs, and was authorized to do and doing business in Galveston, TX.[6]

36.3. Defendant, Beverly Bokovitz, was at all times pertinent, the Chief Nursing Officer and PolicyMaker of Shriners and was aware and responsible for duties owed to Plaintiffs under the organization's FWA, IRB, CDC COVID-19 Vaccination Program Provider Agreement on behalf of Shriners Hospitals for Children. Ms. Bokovitz is named as a defendant in her official and individual capacities.

36.4. Defendant, Frances Farley, MD, was at all times pertinent, the Chief Medical Officer and PolicyMaker of Shriners and was aware and responsible for duties owed to Plaintiffs under the organization's FWA, IRB, CDC COVID-19 Vaccination Program Provider Agreement on behalf of Shriners Hospitals for Children. Dr. Farley is named as a defendant in her official and individual capacities.

36.5. Defendant, Jerry Gantt, was at all times pertinent, the Chairman of the Board of Trustees and PolicyMaker of Shriners Hospitals for Children, is an individual of the full age of

[6] Shriners Hospitals for Children, Inc. and Shriners Hospital for Children – Texas are hereinafter referred to collectively as "Shriners."

majority and a resident of Houston, Texas, and was a signatory to Shriners Hospitals for Children's COVID-19 policy. Mr. Gantt is named as a defendant in his official and individual capacities.

36.6. Defendant, John P. McCabe, was at all times pertinent, the Executive Vice President and Chief Operating Officer and PolicyMaker of Shriners Hospitals for Children, is an individual of the full age of majority and a resident of Florida, and was a signatory to Shriners Hospitals for Children's COVID-19 policy. Mr. McCabe is named as a defendant in his official and individual capacities.

36.7. Defendant, Phillip Grady, was at all times pertinent, the Vice President of Hospital Operations and PolicyMaker of Shriners Hospitals for Children, is an individual of the full age of majority and a resident of Florida, and was a signatory to Shriners Hospitals for Children's COVID-19 policy. Mr. Grady is named as a defendant in his official and individual capacities.

36.8. Defendant, Cecile Erwin Young, is the Executive Commissioner and policymaker of Texas Health and Human Services, a state agency that administers several state programs, including the CDC COVID-19 Vaccination Program, but is sued here solely in her individual capacity.

V. LEGAL BACKGROUND

37. Two federal statutes and one federal program are the gravamen of Plaintiffs' complaints—the EUA statute, PREP Act, and CDC COVID-19 Vaccination Program.

38. **Project Bioshield** (the EUA statute) was enacted into law on July 21, 2004 to authorize expanded access protocols[7] during a declared emergency[8] for drugs, biologics, and

[7] Project Bioshield amended 21 U.S.C. §360bbb-3. 21 U.S.C. § 360bbb *et. seq*., has the short title, "Expanded Access to Unapproved Therapies and Diagnostics."
[8] 21 U.S.C. §360bbb-3(b)

devices not licensed by the FDA for their intended emergency use[9] according to the product's labeling when such products do not exist in the marketplace to meet the intended emergency use.[10]

39. "Unlicensed use" means the use of a medical product for a purpose not licensed by the FDA (e.g., legal indication, usage, and contraindications) according to the product's labeling.

40. "Legal indication" means the FDA-licensed purpose of the drug pursuant to 21 U.S.C. § 355(a) and labeled according to 21 CFR 201.57(c)(2).

41. An investigational drug does not have a legal indication to treat, cure, or prevent any known disease or virus. (See, for example, Exhibit B, FDA's EUA Letter to Pfizer.)

42. Congress only authorizes the Secretary ("Secretary") of the Department of the Health and Human Services ("HHS") to introduce into commerce drugs, biologics, and devices not FDA-licensed for their intended use.[11]

43. The Secretary's authority is nondelegable.

44. An investigational medical product federally funded under the EUA statute is not exempt from the federal government's obligation under 45 C.F.R. §§ 46.122,116 and the Belmont Report.[12],[13]

[9] 21 U.S.C. §§360bbb-3(a)(2)(A)
[10] 21 U.S.C. §360bbb-3(c)(3)
[11] 21 U.S.C. §360bbb-3(a)(1)
[12] Congress enacted The National Research Act ("NRA") in 1974 to prevent medical research abuses by the executive branch, military, and persons acting on behalf of the federal government. The NRA required the Secretary to promulgate 45 C.F.R. Part 46 to protect persons involved in investigational new drugs under research conditions. "Research" under the legal framework is not a clinical trial but an activity "designed to develop or contribute to generalizable knowledge" about the product, theory, process, etc. As an example, college students studying medical charts is "research" or the EUA statute requiring the "manufacturer," "emergency response stakeholder," and "vaccination provider" to "monitor" and "report" adverse events of the drugs' involvement with a human is research because the requirement is "designed to develop or contribute to generalizable knowledge." The Belmont Report, required by the NRA, established the principles of informed consent and is required adherence under 45 C.F.R. Part 46 (§§ 101(c),(i)).
[13] The National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research.– Belmont Report. Washington, DC: U.S. Department of Health and Human Services. 1979.

45. The Secretary must establish "Conditions of Authorization" [14] for persons participating in any activity authorized under the EUA statute.

46. The primary requirement placed upon the Secretary by Congress is to "Appropriate conditions designed to ensure that individuals to whom the product is administered are informed…of the option to accept or refuse administration of the product."[15]

47. The "option" is a right held exclusively by the individual considering the administration of the product in which a third party may not interfere.

48. The Secretary issues an Emergency Use Authorization ("EUA") letter to the manufacturer when its product has been granted an expanded access protocol for the unlicensed use of its product.

49. Within the EUA letter, the Secretary outlines the "Conditions of Authorization" for the product and establishes conditions ensuring individuals are informed of their rights, to which any person participating in any activity under the statute, including administering the product, must comply.

50. At all times pertinent herein, the Secretary required the "emergency response stakeholder" (i.e., state's executive branch) and "vaccination providers" (i.e., Organizations under the Provider Agreement) to ensure individuals were made aware of their right to accept or refuse in each EUA issued since the beginning of the pandemic (see *infra*).[16]

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[14] 21 U.S.C. §360bbb-3(e)

[15] 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)

[16] The Sixth Circuit Court of Appeals erred when claiming "The EUA statute's relevant language—ensur[ing] that individuals to whom the product is administered are informed . . . of [their] option to accept or refuse" the vaccine—addresses the interaction between the medical provider and the person receiving the vaccine, not the interaction between an employer and an employee receiving a vaccine. 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)." *Norris v. Stanley*, 73 F.4th 431, 438 (6th Cir. 2023) First, the EUA statute relies on the Secretary to establish who is responsible for ensuring an individual gives their informed consent. Second, the Secretary performed that function when requiring the state's executive leadership to execute on that requirement within the state. Third, the statute does not require any healthcare provider to obtain informed consent from any individual at any time. There is no language within the statute even

51. If Congress creates a right for an individual, then a third party acting under the color of law is prohibited from depriving that right without due process.

52. The Secretary, not Defendants, establishes the conditions by which an individual uses an EUA/PREP Act drug or device.

53. Congress requires the Secretary to establish appropriate conditions informing individuals of the "health"[17] consequences of taking the product, not taking the product, or ending the therapy early.[18]

54. The term "consequences" in the EUA statute does not contemplate any consequences other than "health" consequences, so the statute does not provide any person with any authority to impose civil, criminal, or other negative consequences, such as termination from employment, on any individual exercising their right to refuse.

55. Congress expressly stated that "No person shall introduce or deliver for introduction into interstate commerce any new drug unless an approval of an application filed pursuant to subsection (b) or (j) of this section is effective with respect to such drug."[19]

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suggesting such a requirement. The practicality of the EUA Statute is that a person either chooses to accept or refuse the product which can be exercised by going to the location where the product is offered or not. The Secretary required the state's executive leadership to ensure individuals were provided an EUA Fact Sheet which informed the individual of their right to either accept or refuse the product. Individuals exercised their right to refuse but the Sixth Circuit held that a third-party had the right to penalize that choice although Congress preempted such a person from having such power.

[17] Exhibit E, FDA Guidance, Appendix A: "**Consequences** of not taking/using [PRODUCT], including possible health effects and quarantine, and of stopping the use of [PRODUCT] against the recommendation of the health care provider." [Emphasis in original.]

[18] The Third Circuit Court of Appeals recently ruled in error stating, "there is no unqualified right to decide whether to 'accept or refuse' an EUA product without consequence. To the contrary, being advised of the consequences is precisely what § 360bbb-3(e)(1)(A)(ii)(III) requires, providing explicitly that the recipient of an EUA product shall be informed 'of the consequences, if any, of refusing administration of the product.'" *Children's Health Defense Inc. v. Rutgers, the State University of New Jersey*, No. 22-2970 (3d Cir. 2024) The ruling cannot reconcile with Congress stating, "Nothing in this section provides the Secretary any authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section" (21 U.S.C. §360bbb-3(l)). Moreover, the Court created a legislative loophole depriving an individual of their legal interest under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) to refuse without consequence. Additionally, the ruling deprives an individual of their fundamental right to refuse an unlicensed investigational new drug without consequence effectively usurping from Congress its authority to prohibit nonconsensual participation in a federally funded investigational drug.

[19] 21 U.S.C. § 355(a)

56. However, for emergency uses, Congress requires the Secretary to inform healthcare professionals "that the Secretary has authorized the emergency use of the product,"[20] which provides notice that healthcare providers can administer the unlicensed product without violating federal drug labeling laws (21 U.S.C. §§ 331(a),355(a).[21]

57. Congress specifically creating the right to "accept or refuse" an EUA investigational product relates to the historical abuses committed by governments, military, and medical researchers, such as those committed against African-American males at the hands of their government in 1972 (Tuskegee Experiment) and thus leaves the choice to the individual.[22]

58. In 1973, Senator Edward Kennedy, enraged by the abuses of the Tuskegee Study, held live hearings in support of passing the National Research Act, exposing medical research abuses[23] by the federal government, military, and pharmaceutical companies, primarily impacting persons of color, women, Indians, and the poor.

59. In 1974, Congress enacted the National Research Act[24] establishing Institutional Review Boards ("IRB"), the Belmont Report,[25] and the requirement of the Secretary to promulgate regulations governing the protection of humans involved in federally funded investigational medical products.

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[20] 21 U.S.C. §360bbb-3(e)(1)(A)(i)(I)

[21] Any person sponsoring, acting on behalf of the sponsor, or administering an investigational new drug to an individual must come under the purview of an Institutional Review Board, 45 C.F.R. Part 46, and its FWA.

[22] Tuskegee Study - Timeline - CDC - OS. Published 2023. Accessed November 23, 2023. https://www.cdc.gov/tuskegee/timeline.htm

[23] Examples of research abuses include (1) US Navy spraying San Francisco with a bacterial agent to study bio warfare (Operation Sea-Spray), (2) Chester Southam of Sloan-Kettering injecting live cancer cells into 300 healthy human female prisoners without informed consent, (3) Saul Krugman of Willowbrook State School in Staten Island, New York, deliberately infecting mentally disabled children with hepatitis by feeding them extracts made from infected feces, (4) the Atomic Energy Commission (AEC) injecting newborns with radioactive iodine in their thyroid glands, (5) full body radiation experiments on the poor, disabled, minorities, and Alaskan Inuit Indians over a period of decades by the AEC.

[24] Title II of the National Research Act, https://www.govinfo.gov/content/pkg/STATUTE-88/pdf/STATUTE-88-Pg342.pdf

[25] The National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research. - Belmont Report. Colorado, DC: U.S. Department of Health and Human Services,1979

60. In the early 1980s, the HHS Secretary published those regulations under 45 C.F.R. Part 46 (Common Rule). Although these are regulations, due to the complexity of ethical biomedical and behavioral research, they were drafted explicitly at Congress's express request to protect humans involved in federally funded research projects.

61. To ensure unambiguous intent, Congress defines "research" as any activity "designed to develop or contribute to generalizable knowledge."[26]

62. Research activities under 45 C.F.R. § 46 are not the same as clinical trials under the FDA by manufacturers seeking a Biologics License Application ("BLA").

63. Moreover, "Activities that meet this definition constitute research for purposes of this policy, whether or not they are conducted or supported under a program that is considered research for other purposes."[27]

64. 45 C.F.R. Part 46 is a requirement of all federal agencies, departments, and the military and persons conducting business with those agencies and departments.[28]

65. The entire federal budget is filtered through 45 C.F.R. § 46.122.[29]

66. The National Research Act specifically commissioned a finding on the nature and legal definition of informed consent.[30]

[26] 45 CFR 46.102(l)
[27] *Id.*
[28] 45 CFR 46.101(a)
[29] "Federal funds administered by a Federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied." 45 C.F.R. 46.122
[30] Title II of the National Research Act, See the requirement of Congress under page 349 (B)(iv) "The nature and definition of informed consent in various research settings."

67. Congress incorporated the findings of the Belmont Report under 45 CFR § 46.116.[31],[32]

68. Legally effective informed consent[33] means, at a minimum, that an individual must be free from all outside pressures to participate in the administration of the federally funded investigational drug, biologic, or device before the offer to participate is presented.[34]

69. Therefore, a sanction for refusing to participate violates the intent of Congress under 45 C.F.R. § 46.116, and all COVID-19 drugs are subject to 45 C.F.R. Part 46 because the executive branch of government purchased them.

70. The only COVID-19 drugs available to Plaintiffs for compliance with the CDC Program were classified by the FDA as investigational, funded by the federal government (DoD funding), authorized under the EUA statute, and covered by the PREP Act.

[31] 45 CFR § 46.101(c) & 45 CFR § 46.101(i) require any federally funded research activity to come under the purview of the Belmont Report and provides the only known definition of what Congress intends as "legally effective informed consent."

[32] "SEC. 205. Within 60 days of the receipt of any recommendation made by the Commission under section 202, the Secretary shall publish it in the Federal Register and provide opportunity for interested persons to submit written data, views, and arguments with respect to such recommendation. The Secretary shall consider the Commission's recommendation and relevant matter submitted with respect to it and, within 180 days of the date of its publication in the Federal Register." "If the Secretary determines that administrative action recommended by the Commission should be undertaken by him, he shall undertake such action as expeditiously as is feasible." -U.S. Congress 1974 National Research Act

[33] See 45 C.F.R. 46.116

[34] The Third Circuit Court of Appeals erred when citing *Johnson v. Brown*, 567 F. Supp. 3d 1230, 1256-57 (D. Or. 2021) which held that "Plaintiffs had informed consent where they retained the option to 'get the vaccine, apply for an exception, or look for employment elsewhere.'" *Children's Health Defense Inc. v. Rutgers, the State University of New Jersey*, No. 22-2970 (3d Cir. 2024). First, legally effective informed consent, when involving federally funded investigational new drugs, means an individual cannot come under outside pressures to participate in the product's administration. Second, no person is required to apply for an exception to have the right to refuse under the EUA Statute. Third, a private employer cannot misrepresent their authority and amend their conditions of authorization established by the Secretary to require that which Congress prohibits—nonconsensual participation as described under 45 C.F.R. § 46.116 and the Belmont Report (see informed consent).

71. Therefore, the federal government was bound to 45 C.F.R. Part 46, Article VII of the ICCPR Treaty,[35] its Federal Wide Assurance[36] ("FWA") program, the Belmont Report, 10 U.S.C. § 980, and the EUA statute when it offered COVID-19 investigational new drugs to humans.

72. Congress ensured that its past efforts to protect humans involved in medical research were not voided during a declared emergency, clearly stating, "Nothing in this section provides the Secretary any authority to require any person[37] to carry out any activity that becomes lawful pursuant to an authorization under this section."[38]

73. Therefore, the Secretary is the only person authorized by Congress to issue expanded access protocols for the unlicensed use of products under the statute. Still, he is not authorized to require a person to manufacture, distribute, store, administer, or receive the product.

74. The secretary's authority is nondelegable. Thus, no other person has the authority to require any person to participate in an EUA product.

75. A vaccine mandate exclusively relying on an EUA product constitutes an arbitrary and capricious requirement lacking the force of law because:

> (1) products under the EUA do not have a legal indication to treat the intended emergency use,

[35] "[N]o one shall be subjected without his free consent to medical or scientific experimentation." The FDA defines medical experimentation as an "experiment is any use of a drug except for the use of a marketed drug in the course of medical practice" (21 CFR 312.3 "Clinical investigation").
[36] The federal government established the FWA to ensure it complied with its lawful obligations to protect humans involved in investigational new drugs. The FWA requires persons participating in federal funds and investigational research activities to assure the federal government that it will comply in whole with 45 C.F.R. Part 46 and specifically the Belmont Report. The federal government issuing a FWA number to an organization means the organization provided a written assurance to HHS that it would comply with the regulatory framework when involved in federally funded research activities such as the CDC COVID-19 Vaccination Program.
[37] "[I]ndividual, partnership, corporation, and association" 21 U.S.C. § 321(e)
[38] 21 U.S.C. §360bbb-3(l)

(2) individuals have the fundamental right to refuse the administration of an investigational medical product,[39],[40]

(3) Congress restricts the Secretary from requiring nonconsensual participation (21 U.S.C. §360bbb-3(l)),

(4) All US States and Territories have FWAs with the federal government assuring compliance with the Belmont Report and 45 C.F.R. Part 46,

(5) treating an investigational drug as a licensed drug under a mandate constitutes a felony under federal misbranding laws (21 U.S.C. § 331),[41]

(6) no vaccination provider (e.g., organization, public clinic, or health practitioner) has the authority to administer an investigational drug without first obtaining legally effective informed consent,

(7) individuals have the fundamental right to refuse under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) which a third-party is preempted from interfering in that right,

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[39] The Supreme Court "has long said that it will recognize those fundamental rights and liberties which are, objectively, deeply rooted in this Nation's history and tradition, and implicit in the concept of ordered liberty, such that neither liberty nor justice would exist if they were sacrificed." *Washington v. Glucksberg*, 521 U.S. 702, 720– 21 (1997) (cleaned up). And it reaffirmed this approach earlier this year. See *Dobbs v. Jackson Women's Health Org*., 142 S. Ct. 2228, 2242, 2246 (2022)." *Golden Glow Tanning Salon, Inc. v. City of Columbus, Miss*., 52 F.4th 974 (5th Cir. 2022) Congress enacted the Federal Food, Drug, and Cosmetic Act (June 25, 1938, ch. 675. § 1, 52 Stat. 1040) prohibiting persons from introducing drugs into commerce before FDA approves the drug for general commercial marketing. Since 1938, it has extensively regulated the pharmaceutical industry and established who, what, when, and how drugs will be introduced to the public. In 1974, Congress took a strong position that no person can require another person to participate in the administration of an investigational new drug, biologic, or device under threat of incurring a penalty or losing a benefit to which they are otherwise entitled by enacting the National Research Act. That Act led to the Belmont Report, 45 C.F.R. § 46, 21 C.F.R. § 312, 21 C.F.R. §§50,56, Article VII ICCPR Treaty, FWA, 10 U.S.C. § 980, 10 U.S.C. §§ 1107,1107(a) demonstrating rights "deeply rooted in the Nation's history" and for this very specific reason Congress created the right to refuse under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) to ensure the fundamental right to refuse an investigational new drug was secured even when a declared emergency exists. Moreover, *Abigail Alliance v. Eschenbach*, 495 F.3d 695 (D.C. Cir. 2007) held that the Secretary alone has authority to establish conditions under which the public can access investigational new drugs and that such authority was deeply rooted in the concepts of liberty for the explicit purpose of protecting humans involved in the investigational use of drugs. The *Abigail* Court held there is no fundamental right to access an investigational new drug, and the Supreme Court denied certiorari.

[40] "Congress has prohibited the administration of investigational drugs to service members without their consent. This Court will not permit the government to circumvent this requirement" *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004)

[41] "Under the provisions of the Food, Drug and Cosmetic Act, a company must specify the intended uses of a product in its new drug application to FDA. Once approved, the drug may not be marketed or promoted for so-called 'off-label' uses – i.e., any use not specified in an application and approved by FDA. Pfizer promoted the sale of Bextra for several uses and dosages that the FDA specifically declined to approve due to safety concerns. The company will pay a criminal fine of $1.195 billion, the largest criminal fine ever imposed in the United States for any matter." - Justice Department Announces Largest Health Care Fraud Settlement in Its History. Justice.gov. Published September 2, 2009. Accessed November 12, 2023. https://www.justice.gov/opa/pr/justice-department-announces-largest-health-care-fraud-settlement-its-history

> (8) no person outside of the Secretary has authority to authorize an EUA nor amend an existing one to require that which Congress prohibits—nonconsensual participation.

76. The Secretary may, under discretionary authority, grant expanded access to the unlicensed use of products under the EUA statute.

77. Still, the Secretary only has ministerial duties to ensure that no person is required to manufacture, distribute, store, administer, or receive any product authorized pursuant to the Statute.

78. This restriction ensures that individuals are not under outside pressure to participate even during a declared emergency, thus requiring an individual's legally effective informed consent to be obtained.[42]

79. **Preemption.** Congress expressly crafted language preempting state and local law conflicting with the PREP Act's voluntary requirement,[43] which provides, in pertinent part:

> (8) Preemption of State law
>
> "During the effective period of a declaration under subsection (b)…no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that— (A) is different from, or is in conflict with, any requirement applicable under this section; and (B) relates to the…administration…of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act." ("FDCA")[44] (emphasis added).

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[42] The first EUA issued under the statute was for Anthrax and made available to service members and civilians. The Secretary required the following information under the EUA: (1) Individuals (service members and civilians) who refuse anthrax vaccination will not be punished. (Emphasis added), (2) Refusal may not be grounds for any disciplinary action under the Uniform Code of Military Justice, (3) Refusal may not be grounds for any adverse personnel action. Nor would either military or civilian personnel be considered non-deployable or processed for separation based on refusal of anthrax vaccination, (4) There may be no penalty or loss of entitlement for refusing anthrax vaccination, (5) This information shall read in the trifold brochure provided to potential vaccine recipients as follows: You may refuse anthrax vaccination under the EUA, and you will not be punished. No disciplinary action or adverse personnel action will be taken. You will not be processed for separation, and you will still be deployable. There will be no penalty or loss of entitlement for refusing anthrax vaccination. The reason the HHS required this information was to ensure legally effective informed consent was obtained.

[43] 42 U.S.C. § 247d-6d(b)(8)

[44] 21 U.S.C. § 301 *et. seq*.

80. The option to accept or refuse under the EUA statute is located at 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III), which is found within the FDCA.

81. Thus, any State legal requirement that conflicts with the provisions of the EUA statute is preempted by the PREP Act because the drugs mandated by Defendants were also listed as countermeasures under the PREP Act.

82. Therefore, Congress expressly preempted Defendants from "establishing, enforcing, or continuing in effect" with a legal requirement (i.e., mandate or state custom of penalizing individuals who refused the EUA drugs) requiring nonconsensual use of an EUA product that was also a PREP Act covered countermeasure as a condition of enjoying a privilege of the State (e.g., welfare, unemployment, education, public employment, state-issued license) because it "is different from, or conflicts with" requirements under the EUA statute and the PREP Act.

83. The private employer continues in effect with a state's at-will employment doctrine when the employer terminates (e.g., amending or ending employment) an employee for refusing participation in a voluntary PREP Act program.

84. Thus, any court applying state law is preempted from applying a state's at-will employment doctrine in a way that conflicts with the EUA statute or the PREP Act.

85. Similarly, a state government that issues a requirement to use an EUA product establishes a legal requirement that conflicts with the EUA statute and the PREP Act.

86. Under the Field Preemption test, such a legal requirement violates the Supremacy Clause doctrine.[45]

[45] *Arizona v. United States*, 567 U.S. 387, 399 (2012) ("[Congress's] intent to displace state law altogether can be inferred from a framework of regulation 'so pervasive . . . that Congress left no room for the States to supplement it'

87. The federal government's regulations are so pervasive that they exclusively occupy the drug classification (FDA) field, providing expanded access protocols and establishing conditions for the unlicensed use of drugs and devices (HHS).[46]

88. No private hospital can authorize the use of or administer an unlicensed COVID-19 EUA drug without federal governmental authority.

89. Congress preempts a private employer from requiring mandatory use of an EUA drug and using the state's at-will employment law to retaliate against employees who exercise their option to refuse.

90. Congress completely dominates the EUA field and the use of a state's at-will employment law to penalize those who refuse conflicts with the federal goals of the EUA.

91. Therefore, any mandate issued by a public or private entity placing individuals under threat of penalty for refusing an EUA product conflicts with the federal goals and cannot reconcile with the product's lawful obligations established by valid acts of Congress.[47]

92. No person has constitutional, statutory, common law, or other authority to require an individual to inject an unlicensed investigational drug into their body as a condition of enjoying a privilege of the State (e.g., welfare, unemployment, education, public employment, state-issued license) and Defendants cannot prove differently.

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or where there is a 'federal interest . . . so dominant that the federal system will be assumed to preclude enforcement of state laws on the same subject.'") (quoting *Rice v. Santa Fe Elevator Corp*., 331 U.S. 218, 230 (1947))

[46] *Rice v. Santa Fe Elevator Corp*., 331 U.S. 218, 230 (1947)

[47] The Third Circuit Court of Appeals erroneously held that the Preemption Doctrine did not apply to Rutgers vaccine policy stating "First, § 360bbb-3(e)(1)(A) obligates only the Secretary of Health and Human Services to act, by establishing "conditions designed to ensure" informed consent. Because Section 360bbb-3(e)(1)(A) does not impose any obligations on state universities, it cannot conflict with Rutgers' Policy." First, the Statue imposes the obligation upon Rutgers not to interfere in an individual's autonomous chosen option. Second, it is not that the statute conflicts with Rutgers policy, it is that Rutgers policy conflicts with the federal goal of ensuring only free consent of persons participating in any activity authorized by the statute. Third, Rutgers committed an act of fraud by misrepresenting their authority when issuing a vaccine mandate effectively amending the EUA as enacted by the Secretary. *Children's Health Defense Inc. v. Rutgers, the State University of New Jersey*, No. 22-2970 (3d Cir. 2024)

93. **PREP Act.** The PREP Act was enacted on December 30, 2005, and codified at 42 U.S.C. § 247d-6d and 42 U.S.C. § 247d-6e, respectively.

94. The PREP Act is a "program" providing immunities for "covered countermeasures,"[48] "covered persons,"[49] "covered individuals,"[50] and "qualified persons"[51] volunteering to participate in activities or countermeasures authorized under the statute.

95. The PREP Act, by its express terms, requires individuals to consent to forfeit their right to seek judicial relief voluntarily should they incur an injury from involvement in any activity or the administration of PREP Act countermeasures.

96. The forfeiture of an individual's right to seek judicial relief for injuries is why Congress mandates:

> "The Secretary **shall ensure** that a State, local, or Department of Health and Human Services plan to administer or use a covered countermeasure is consistent with any declaration under 247d–6d of this title…and that potential **participants are educated** with respect to contraindications, **the voluntary nature of the program**, and the availability of potential benefits and compensation under this part (Emphasis added).[52]

97. Regarding the instant action, the EUA statute's "required condition…that individuals…are informed…of the option to accept or refuse" is a "requirement applicable to the covered countermeasure" because all of the EUA drugs available to Plaintiffs were listed as countermeasures under the PREP Act.[53]

[48] 42 USC § 247d-6d(i)(1)
[49] 42 USC § 247d-6d(i)(2)
[50] 42 USC 247d-6d(a)(3)(C)(i,ii)
[51] 42 USC § 247d-6d(i)(8)
[52] 42 U.S.C. § 247d-6e(c)
[53] 21 U.S.C. § 360bbb-3(e)(1)(A)(ii)(III)

98. The use of laws to penalize those who refuse a PREP Act countermeasure conflicts with the voluntary nature of the PREP Act program and interferes with the option to refuse under the EUA statute.

99. Therefore, should an individual incur an injury from the compulsory use of the product, they would not have recourse against the manufacturer or person administering the product.

100. No person has constitutional, statutory, common law or other authority to require an individual to inject any drug, licensed or otherwise, covered under the PREP Act into their body as a condition of enjoying a privilege of the State (e.g., welfare, unemployment, education, public employment, state-issued license) and Defendants cannot prove differently.

101. The State establishes an unconstitutional condition when placing an individual under a sanction refusing participation because the individual is required to "barter away his life or his freedom, or his substantial rights"[54] to due process under the PREP Act as a condition of enjoying a privilege of the State (e.g., welfare, unemployment, education, public employment, state-issued license).

102. **CDC Program.** The federal government purchased the COVID-19 drugs using DoD funding authority,[55] requiring the Biden Administration to comply with 10 U.S.C. § 980.[56]

[54] *Home Ins. Co. of New York v. Morse*, 87 U.S. 455, 451 (1874)
[55] See Exhibit F, DoD Purchase of COVID-19 Drugs
[56] "Funds appropriated to the Department of Defense may not be used for research involving a human being as an experimental subject unless— (1) the informed consent of the subject is obtained in advance; or (2) in the case of research intended to be beneficial to the subject, the informed consent of the subject or a legal representative of the subject is obtained in advance." Any person acting on behalf of the sponsor (i.e., federal government) when administering the COVID-19 EUA drugs under research conditions is bound by this statute to obtain the legally effective informed consent of any individual under their authority. Placing an individual under threat of penalty nullifies legally effective informed consent even if the individual agrees to participate in the product/program/activity per 45 C.F.R. § 46.116 and the Belmont Report.

103. Moreover, the drugs were under EUA and classified by the FDA as investigational, requiring the executive branch of the federal government to comply with 45 C.F.R. Part 46, its FWA program, 21 U.S.C. §360bbb-3, Article VII ICCPR Treaty, 10 U.S.C. § 980, the Belmont Report, among other legal requirements.[57],[58]

104. The federal government established the CDC COVID-19 Vaccination Program to distribute, administer, monitor, and obtain data regarding the COVID-19 EUA/PREP Act drugs. It required that distribution be accomplished through each state's Immunization Information System (IIS) via a partnership with private-party state actors that administered the injections.

105. The federal government distributed the drugs through each state's IIS because each state had an existing FWA whereby the state assured the federal government it would comply with 45 C.F.R. Part 46 and the Belmont Report anytime it offered federally funded investigational medical products to a human being.

106. The CDC published its "COVID-19 Vaccination Program Interim Operational Guidance Jurisdiction Operations"[59] ("Playbook") playbook.

107. The Playbook was designed to help states "operationalize a vaccination response to COVID-19 within their jurisdictions" and help states with their "COVID-19 vaccination program planning and implementation."

108. States desiring to participate were to use the Playbook to help it administer the CDC Program.

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[57] See Exhibit B, FDA's August 23, 2021, EUA Letter to Pfizer

[58] The Third Circuit Court of Appeals stated, "But there is no constitutional right either to refuse vaccination, *Jacobson*, 197 U.S. at 31." *Children's Health Defense Inc. v. Rutgers, the State University of New Jersey*, No. 22-2970 (3d Cir. 2024) The Court erred because it applied its ruling to the use of investigational drugs under EUA. The *Jacobson* Court did not have laws restricting persons from mandating the use of investigational drugs nor was there a specific right created by Congress affirming an individual's authority to refuse the product without consequence (21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)). The ruling cannot reconcile with current acts of Congress.

[59] See Exhibit G, CDC Playbook

109. The Playbook informed the State of its duties before any COVID-19 drug was distributed within the State's jurisdiction.

110. The Playbook informed the State that:

> (1) "Vaccination provider" refers to any facility, organization, or healthcare provider licensed to possess/administer vaccine or provide vaccination services. A "COVID-19 vaccination provider" is any vaccination provider who has been enrolled in the COVID-19 Vaccination Program.[60]
>
> (2) "Jurisdiction/jurisdictional," as used in this document, refers to the federal immunization funding awardees described in the Executive Summary and their state public health emergency preparedness counterparts who are tasked with developing COVID-19 vaccination plans for submission to CDC.[61]
>
> (3) To receive/administer COVID-19 vaccine, constituent products, and ancillary supplies, vaccination provider facilities/organizations must enroll in the federal COVID-19 Vaccination Program coordinated through their jurisdiction's immunization program.[62]
>
> > (a) the vaccination provider must agree to the Provider Agreement whereby they promise to "Comply with FDA's requirements, including EUA-related requirements **described in FDA's Letter of Authorization**, as applicable. Providers **must also administer COVID-19 vaccine in compliance with all applicable state and territorial vaccine laws** (emphasis added)."[63]
>
> (4) Enrolled COVID-19 vaccination providers must be credentialed/licensed in the jurisdiction where vaccination takes place, and sign and agree to the conditions in the CDC COVID-19 Vaccination Program Provider Agreement.[64]
>
> (5) Jurisdictions must facilitate and monitor IIS reporting by enrolled vaccination providers.[65]

[60] See Exhibit G, CDC Playbook, Footnote 1
[61] See Exhibit G, CDC Playbook, Footnote 2
[62] See Exhibit G, CDC Playbook, p. 21
[63] The State incorporated the Provider Agreement as its policy when requiring each public or private party to sign and agree to the terms of the CDC Program as a condition to engage in the joint conduct of administering the federally owned COVID-19 drugs on the state's behalf.
[64] See Exhibit G, CDC Playbook, p. 21
[65] See Exhibit G, CDC Playbook, p. 35

> (6) State-level personnel must closely monitor activities at the local level **to ensure** the COVID-19 Vaccination Program is implemented throughout the jurisdiction in adherence with federal guidance and requirements. [66] (emphasis added)
>
> (7) Help the public to understand key differences in FDA emergency use authorization and FDA approval (i.e., licensure).[67]
>
> (8) Jurisdictions will be provided an opportunity to opt out of having pharmacies in their area receive direct allocations.[68]
>
> (9) Ensure provider agreement, profile form, and redistribution agreement (if applicable) are thoroughly and accurately completed by each enrolled provider, retained on file for a minimum of 3 years, and made available to CDC upon request.[69]

111. The Playbook demonstrates the extensive and intimate involvement of the State in the CDC Program and how the State was fully responsible for the implementation and enforcement of the CDC Program.

112. The CDC informed the states:

> "At this time, **all COVID-19 vaccine in the United States has been purchased by the U.S. government** (USG) for administration exclusively by providers enrolled in the CDC COVID-19 Vaccination Program and remains U.S. government property until administered to the recipient. Only healthcare professionals enrolled through a health practice or organization as vaccination providers in the CDC COVID-19 Vaccination Program (and authorized entities engaged in shipment for the Program) are authorized to lawfully possess, distribute, deliver, administer, receive shipments of, or use USG-purchased COVID-19 vaccine. Other possession, distribution, delivery, administration, shipment receipt, or use of COVID-19 vaccine outside the parameters of the Program constitutes, at a minimum, theft under 18 U.S.C. § 641, and violation of other federal civil and criminal laws. Violators are subject to prosecution to the full extent of the law."

113. "Other administration" includes placing an individual under outside pressures to participate in the product's administration outside of their free will and voluntary consent.

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[66] See Exhibit G, CDC Playbook, p. 8
[67] See Exhibit G, CDC Playbook, p. 42
[68] See Exhibit G, CDC Playbook, p. 26
[69] See Exhibit G, CDC Playbook, p. 22

114. Because the CDC Program is a federal program fully funded by the DoD, Medicare, and other federal agencies, it means that the federal government owes constitutional obligations to individuals involved in the program.[70]

115. The federal government cannot delegate the function of administering investigational drugs under the EUA statute and PREP Act without also delegating its constitutional obligations.

116. The Provider Agreement[71] required the State and recruited parties to:

> (1) Organization must administer COVID-19 Vaccine in accordance with all requirements and recommendations of CDC and CDC's Advisory Committee on Immunization Practices (ACIP).
>
> (2) Organization must not sell or seek reimbursement for COVID-19 Vaccine and any adjuvant, syringes, needles, or other constituent products and ancillary supplies that the federal government provides without cost to Organization.
>
> (3) Organization must administer COVID-19 Vaccine regardless of the vaccine recipient's ability to pay COVID-19 Vaccine administration fees.
>
> (4) Before administering COVID-19 Vaccine, Organization must provide an approved Emergency Use Authorization (EUA) fact sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative.[72]
>
> (5) Organization must report moderate and severe adverse events following vaccination to the Vaccine Adverse Event Reporting System (VAERS).[73]
>
> (6) Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine.

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[70] *Biden v. Missouri*, 595 U.S. 87,93 (2022) was a facial challenge to the President's authority to issue a vaccine mandate for healthcare facilities within certain parameters, which the Court upheld. However, the Court neither considered nor ruled on how compliance with President Biden's mandate could only be achieved through the use of federally funded investigational drugs which the Court would have struck down immediately given the significant statutory and constitutional obligations owed to individuals involved with federally funded investigational new drugs and applicable case law.

[71] See Exhibit A, Provider Agreement

[72] The Fact Sheet serves the purpose of obtaining an individual's legally effective informed consent.

[73] This "reporting" constitutes "research" under 45 C.F.R. Part 46 and the Belmont Report.

> (7) Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws.[74]

117. The CDC further informed the State that:

> "The above requirements are material conditions of payment for COVID-19 Vaccine-administration claims submitted by Organization to any federal healthcare benefit program, including but not limited to Medicare and Medicaid, or submitted to any HHS-sponsored COVID-19 relief program, including the Health Resources & Services Administration COVID-19 Uninsured Program. Reimbursement for administering COVID-19 Vaccine is not available under any federal healthcare program if Organization fails to comply with these requirements with respect to the administered COVID-19 Vaccine dose. Each time Organization submits a reimbursement claim for COVID-19 Vaccine administration to any federal healthcare program, Organization expressly certifies that it has complied with these requirements with respect to that administered dose" and "Non-compliance with the terms of Agreement may result in suspension or termination from the CDC COVID-19 Vaccination Program and criminal and civil penalties under federal law, including but not limited to the False Claims Act, 31 U.S.C. § 3729 *et seq*., and other related federal laws, 18 U.S.C. §§ 1001, 1035, 1347, 1349."

118. The State assumed the responsibility of the laws, regulations, and other legal requirements when volunteering to use its IIS to administer the CDC COVID-19 emergency program, which was an exclusive function of the federal government and, by direct proximity, became an exclusive function of the state government.

119. Having assumed the obligation of administering the federal property under the EUA statute and the PREP Act, the State owed Fourteenth Amendment obligations to all persons within its jurisdiction.

120. The Provider Agreement was unambiguous that the "Organization must comply with all applicable requirements set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine."

[74] The Provider Agreement is not only an agreement between the "Organization" and the federal government, but also an agreement between the Organization and the State since the State incorporated the Provider Agreement requirements into State policy.

121. The EUA statute is at the heart of Plaintiffs' claims, providing them the right to accept or refuse EUA drugs without threat of penalty.[75]

122. Therefore, when the State agreed to administer the CDC Program within the State's jurisdiction, it assumed the duty that "State-level personnel must closely monitor activities at the local level to ensure the COVID-19 Vaccination Program is implemented throughout the jurisdiction **in adherence with federal guidance and requirements.**"[76]

123. The State violated the Equal Protection Doctrine when it treated an individual who accepted the administration of the EUA drugs differently than an individual who refused because both individuals were similarly situated, chose an option under the same provision of law, and were treated unequally as a result.[77] Such discrimination did not "adhere[] with federal guidance and requirements."

124. The State also could not deny a public benefit (e.g., unemployment, education, use of a state-issued medical license, etc.) to individuals who refused but not those who accepted without violating the Equal Protection Doctrine.

125. Lastly, the State could not allow parties recruited to act on the State's behalf to treat individuals differently based on their chosen option without violating the Equal Protection Doctrine.

126. No person within the State of Texas could administer the product without the State's authorization.

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[75] The Supreme Court held that when a federal statute contains rights creating language for an individual then that right is enforceable under 42 U.S.C. § 1983 - *Health & Hosp. Corp. of Marion Cnty. v. Talevski*, 599 U.S. 166, 183 (2023)

[76] See Exhibit G, CDC Playbook, p. 22

[77] The State allowed healthcare providers to destroy decades long careers for choosing the option to refuse. The State denied unemployment to persons refusing the federally funded COVID-19 EUA/PREP Act products. The State allowed public institutions to mandate the use of the federal property under threat of penalty nullifying individuals' rights under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III).

127. **EUA Letter.** The Secretary activated the EUA statute by issuing EUA letters to the manufacturers of products authorized for unlicensed emergency use.

128. An EUA letter stipulates the "Scope of Authorization," "Conditions of Authorization," and assigned duties for persons and entities volunteering to participate in the authorized activities.

129. Since the beginning of the pandemic, the Secretary has issued and reissued dozens of EUAs to Pfizer, Moderna, and Johnson & Johnson for their COVID-19 investigational drugs, identifying assigned duties for the manufacturer, emergency response stakeholder, and vaccination provider.

130. For example, the Secretary issued EUAs for the (1) Pfizer-BioNTech COVID-19 Vaccine on December 11, 2020, stating that "Pfizer-BioNTech COVID-19 Vaccine" "is an investigational vaccine not licensed for any indication,"[78] (2) "Moderna COVID-19 Vaccine" on December 18, 2020 stating that "Moderna COVID-19 Vaccine" "is an investigational vaccine not licensed for any indication,"[79] and (3) "Janssen COVID-19 Vaccine" on February 27, 2021, Inc., stating that "Janssen COVID-19 Vaccine" "is an investigational vaccine not licensed for any indication."[80]

131. Attached as Exhibit B is the August 23, 2021 reissuance of the EUA for the Pfizer-BioNTech COVID-19 Vaccine drug.[81]

[78] 86 Fed.Reg. 5200, Jan. 19, 2021
[79] 86 Fed.Reg. 5200, Jan. 19, 2021
[80] 86 Fed.Reg. 28608, May 27, 2021
[81] The FDA improperly allowed Pfizer to include the word "Vaccine" within its investigational formal name to give the appearance that the drug was classified as a vaccine. The capital "V" denotes the formal name for Pfizer's unlicensed investigational drug "Pfizer-BioNTech COVID-19 Vaccine." "Pfizer-BioNTech COVID-19 vaccine" (lowercase v) denotes Pfizer's COVID-19 vaccine technology and is not attributed to any specific drug label. COMIRNATY® is the only Pfizer COVID-19 drug licensed by the FDA for general commercial marketing and was not available at all times pertinent to this Amended Complaint.

132. Section III of the EUA letter assigns duties to Pfizer, emergency response stakeholders (i.e., State executive leadership), and vaccination providers (i.e., "Organizations" under the Provider Agreement, which are the parties the State authorized).

133. Pfizer is under a duty to:

> (1) "Pfizer Inc. will ensure that the terms of this EUA are made available to all relevant stakeholders (e.g., emergency response stakeholders, authorized distributors, and vaccination providers) involved in distributing or receiving authorized Pfizer- BioNTech COVID-19 Vaccine. Pfizer Inc. will provide to all relevant stakeholders a copy of this letter of authorization and communicate any subsequent amendments that might be made to this letter of authorization and its authorized labeling."
>
> (2) "Pfizer Inc. will report to Vaccine Adverse Event Reporting System (VAERS): Serious adverse events (irrespective of attribution to vaccination); Cases of Multisystem Inflammatory Syndrome in children and adults; and Cases of COVID-19 that result in hospitalization or death, that are reported to Pfizer Inc."
>
> (3) "Pfizer Inc. must submit to Investigational New Drug application (IND) number 19736" for its Pfizer-BioNTech COVID-19 Vaccine drug.
>
> (4) "Pfizer Inc. will conduct post-authorization observational studies to evaluate the association between Pfizer-BioNTech COVID-19 Vaccine and a pre-specified list of adverse events of special interest, along with deaths and hospitalizations, and severe COVID-19," denoting research activities under 45 C.F.R. Part 46, Pfizer's IRB, and its FWA.

134. The Emergency Response Stakeholder (i.e., State) is under the duties of the CDC Program as a whole but specific to each EUA, the State must:

> (1) "Emergency response stake holders will identify vaccination sites to receive authorized Pfizer-BioNTech COVID-19 Vaccine **and ensure its** distribution and **administration**, **consistent** with the terms of this letter and CDC's COVID-19 Vaccination Program" (emphasis added). This duty upon the State denotes an ongoing ministerial duty to perform.
>
> (2) "Emergency response stakeholders will ensure that vaccination providers within their jurisdictions are aware of this letter of authorization, and the terms herein and any subsequent amendments that might be made to the letter of authorization, **instruct them about the means** through which they are **to obtain and administer the vaccine under the EUA**, and

> ensure that the authorized labeling [i.e., Fact Sheet for Healthcare Providers Administering Vaccine (Vaccination Providers) and Vaccine Information Fact Sheet for Recipients and Caregivers] is made available to vaccination providers through appropriate means (e.g., e-mail, website)" (emphasis added). This duty requires the State to "instruct" parties they recruit of the lawful administration of the EUA product which, at the bare minimum, must include the requirement to ensure individuals are made aware of their lawful right to accept or refuse without incurring a penalty or losing a benefit to which the individual is otherwise entitled.

135. The Vaccination Provider (i.e, Shriners Defendants) has duties assigned under the Provider Agreement but specific to each EUA they must:

> (1) "Vaccination providers will administer the vaccine in accordance with the authorization and will participate and comply with the terms and training required by CDC's COVID-19 Vaccination Program."
>
> (2) "Vaccination providers will provide the Vaccine Information Fact Sheet for Recipients and Caregivers to each individual receiving vaccination and provide the necessary information for receiving their second dose and/or third dose."
>
> (3) "Vaccination providers administering the vaccine must report the following information associated with the administration of the vaccine of which they become aware to VAERS in accordance with the Fact Sheet for Healthcare Providers Administering Vaccine (Vaccination Providers): Vaccine administration errors whether or not associated with an adverse event; Serious adverse events (irrespective of attribution to vaccination); Cases of Multisystem Inflammatory Syndrome in children and adults; Cases of COVID-19 that result in hospitalization or death." These reporting requirements denote research activities required of the federal government and the State.
>
> (4) "Vaccination providers will conduct any follow-up requested by the U.S government, including CDC, FDA, or other designee, regarding adverse events to the extent feasible given the emergency circumstances. Vaccination providers will monitor and comply with CDC and/or emergency response stakeholder vaccine management requirements (e.g., requirements concerning obtaining, tracking, and handling vaccine) and with requirements concerning reporting of vaccine administration data to CDC. Vaccination providers will ensure that any records associated with this EUA are maintained until notified by FDA. Such records will be made available to CDC, and FDA for inspection upon request."

136. Duties placed upon all persons participating in the Pfizer-BioNTech COVID-19 Vaccine EUA drug is:

> (1) "All descriptive printed matter, advertising, and promotional material, relating to the use of the Pfizer-BioNTech COVID-19 Vaccine shall be consistent with the authorized labeling, as well as the terms set forth in this EUA, and meet the requirements set forth in section 502(a) and (n) of the FD&C Act and FDA implementing regulations." Therefore, any vaccine mandate relying exclusively on the EUA drug must ensure its "printed matter" reflects the requirements under the "FDA implementing regulations" and "consistent with the authorized labeling." Therefore, should a person issue a requirement to inject the drug under threat of penalty then such promotional material would not be consistent with the "authorized labeling" or "FDA implementing regulations."
>
> (2) "All descriptive printed matter, advertising, and promotional material relating to the use of the Pfizer-BioNTech COVID-19 Vaccine clearly and conspicuously shall state that: This product has not been approved or licensed by FDA, but has been authorized for emergency use by FDA, under an EUA to prevent Coronavirus Disease 2019 (COVID-19) for use in individuals 12 years of age and older; and The emergency use of this product is only authorized for the duration of the declaration that circumstances exist justifying the authorization of emergency use of the medical product under Section 564(b)(1) of the FD&C Act unless the declaration is terminated or authorization revoked sooner."

137. The duties of the State and Organizations under the CDC Program are contractual. The duties of the State and Organizations under the EUA statute are federal, legal, and statutory requirements. Both are legal requirements for any person involved in administering the federally funded COVID-19 EUA/PREP Act investigational drugs.

138. **The President's Constitutional Duties.** The President purchased the COVID-19 EUA/PREP Act drugs but was required by the Constitution to ensure that no person is "deprived of life, liberty or property without due process of law" under the Fifth Amendment when accepting or refusing the federally owned COVID-19 EUA/PREP Act drugs.

139. The President was also required to protect the right to privacy.[82] Participating in the administration of an investigational drug is a private matter, and the federal government cannot compel an individual to divulge whether they participated in such activity as a condition to enjoy a federal benefit.

140. The President was also required to protect PREP Act rights.

141. The PREP Act requires a person to forfeit their right to seek judicial relief should they sustain an injury due to participating in PREP Act activities and using PREP Act countermeasures.

142. Such a forfeiture of rights must be voluntary, not compulsory.

143. The federal government cannot compel individuals to surrender their Fifth Amendment rights under the PREP Act as a condition of public employment, education, or medical services.

144. The federal government cannot compel nonconsensual participation in any PREP Act activity or nonconsensual use of any PREP Act countermeasure under any circumstances without violating the Fifth Amendment's Due Process Clause.

145. **The State's Constitutional Duties.** Volunteering to administer the CDC Program, the State owes Fourteenth Amendment obligations to individuals who freely agree to receive the drugs administered under the Program.

146. Equal Protection. The State cannot treat an individual refusing the administration of an EUA/PREP Act drug differently from an individual accepting it.

147. The State is under explicit statutory and contractual requirements to ensure individuals choosing the option to accept are treated equally to those choosing to refuse.

[82] *Griswold v. Connecticut*, 381 U.S. 479 (1965)

148. Due Process. The State cannot place an individual under threat of penalty to use an EUA/PREP Act drug.

149. Such a requirement strips individuals of their legal interest under the EUA statute, violating their due process rights.

150. The State cannot require a person to use a PREP Act countermeasure as a condition of receiving a benefit of the State (e.g., welfare, unemployment, education, state-issued license, etc.) because such requirement establishes an unconstitutional condition and violates the substantive due process rights of the individual.

151. **Privacy.** The State cannot compel individuals to divulge their private identifiable information to unknown persons for unknown reasons, which is a requirement under the current EUAs, and involuntary participation in that requirement is an invasion of one's privacy.

152. By requiring nonconsensual participation in a PREP Act program, the State is compelling individuals to divulge their private identifiable information to unknown persons for unknown reasons.

153. The federal government, the State, emergency response stakeholders, vaccination providers, and any person involved in the administration of the product have only a ministerial duty under the EUA statute: offer the product; inform of the right to accept or refuse; provide a Fact Sheet; accept the individual's chosen option.[83]

154. Those involved in the administration of the product have no discretionary authority to act beyond accepting a person's choice.[84]

[83] The Ninth Circuit held that officials were only engaged in ministerial acts unprotected from qualified immunity when refusing to allow plaintiffs to apply for a license. *Groten v. California*, 251 F.3d 844 (9th Cir. 2001)
[84] 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)

155. Such restrictions protect the Equal Protection, Due Process, and Privacy rights of potential EUA/PREP Act drug recipients.

156. At all times pertinent, the State agreed to conduct the research activities outlined in the Provider Agreement and each EUA letter on behalf of the federal government.

157. When the executive branch purchased the EUA/PREP Act drugs, it activated 45 C.F.R. Part 46 and the State's FWA.

VI. FACTUAL ALLEGATIONS

158. At all times pertinent, Plaintiffs were healthcare workers licensed by the state and formerly employed by Shriners Hospitals for Children.

159. Plaintiffs make no assertions regarding whether it is lawful for a public or private entity to mandate the use of a ***licensed*** vaccine. Plaintiffs' allegations herein only relate to Shriners PolicyMakers applying its Shiners Policy to the compulsory use of drugs, biologics, and devices under 21 U.S.C. §360bbb-3 (the EUA statute) and The PREP Act and administered under the State's CDC Program. Plaintiffs' claims contest how Shriners PolicyMakers applied their mandate, not that they issued it.

160. At all times pertinent, the only COVID-19 drugs available to Plaintiffs for compliance with Shriners Policy were wholly owned by the federal government, FDA-classified as investigational, distributed through the State's CDC Program, and only authorized under 21 U.S.C. §360bbb-3 and covered under the PREP Act.

161. Plaintiffs adamantly assert that an individual has the absolute Constitutional and federal statutory right to refuse the administration of an Emergency Use Authorization (EUA) or Emergency Use Instructions (EUI) (see *infra*) drug (e.g., Pfizer BioNTech COVID-19 Vaccine), biologic, or device (e.g., EUA testing articles and masks), without incurring a penalty or losing a

benefit to which they are otherwise entitled. Moreover, such a right is not dependent upon a person seeking a religious or medical exemption.

162. Plaintiffs assert that they have the fundamental right to refuse the administration of an investigational new drug without incurring a penalty or losing a benefit to which they are otherwise entitled because that right is "deeply rooted in [our] history and tradition" and it is essential to this Nation's "scheme of ordered liberty." *Timbs v. Indiana*, 586 U. S. ___, 139 S.Ct. 682, 203 L.E.2d 11 (2019) (internal quotation marks omitted).

163.. Plaintiffs assert they have the Constitutional and federal statutory right to refuse participation in any activity or use of any product (e.g., COMIRNATY® 2023-2024, Spikevax, Novavax) covered under the PREP Act.

161. Plaintiffs assert they have the Fourteenth Amendment right to have the laws involving unlicensed drugs authorized for emergency use applied to them equally and by due process.

162. **The State.** The State undertook the affirmative act of volunteering to perform for the federal government under the CDC Program.

163. The CDC Program required the State to instruct, monitor, and enforce the provisions of the CDC Program, given the special laws about the administration of drugs and devices not licensed for any legal indication nor approved by the FDA for general commercial marketing.

164. The State promised to ensure its recruited parties signed the Provider Agreement, instruct the recruited parties of the requirements of any EUA letter, provide an EUA drug Fact Sheet to each person considering the use of the drug, and perform the ministerial act of accepting an individual's option to accept or refuse under the EUA statute.

165. The option to accept or refuse became a property interest of individuals under the State's authority who were constitutionally obligated to ensure it was administered according to Equal Protection and Due Process.

166. Once the State provided the benefit of considering participation in the federal CDC Program and making an autonomous choice of accepting or refusing, the State was required to ensure both options were administered fairly.[85]

167. When a state promises to provide a benefit under explicit rules, laws, and obligations and then goes back on that promise, it acts arbitrarily, violating the Due Process Clause of the Fourteenth Amendment.[86]

168. The State could not escape its constitutional obligations by delegating its function to private parties without also delegating its constitutional obligations.

169. The State and all recruited parties were engaged in State Action owing constitutional obligations under the Fourteenth Amendment to every person under the State's jurisdiction.

170. The State affirmed it would perform the ministerial act of accepting a person's autonomous choice under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III). However, it then allowed its recruited parties to deprive Plaintiffs of their property interest without procedural due process.

171. Worse yet, the State, having promised to protect that liberty interest, willfully violated its promise by denying healthcare workers unemployment benefits after the State Actors unlawfully discharged them from employment for no other reason than exercising their lawful right to refuse.

172. The State allowed its State Actors to deprive any person of their property interest

[85] *Goldberg v. Kelly*, 397 U.S. 254 (1970)
[86] *Board of Regents of State Colleges v. Roth*, 408 U.S. 564 (1972)

to refuse after considering participation without due process and in violation of the equal protection clause of the Fourteenth Amendment.

173. The historic burning of the Constitutional rights among Texans by its government directly relates to Cecile Erwin Young engaging in reckless behavior and acting with deliberate indifference to the rights, safety, and health of the entire healthcare industry within the State.

A. Defendant Cecile Erwin Young

174. Cecile Erwin Young ("Ms. Young") was appointed executive commissioner of the Texas Health and Human Services Commission (HHSC) by Governor Abbott on Aug. 14, 2020.

175. Ms. Young was not under a legal obligation to assume the duties of that office, but she assumed those duties willfully and took the required oath[87] under TEX. CONST. Art. XVI, §1 swearing to "faithfully execute the duties of the office" as well as to preserve, protect, and defend the Constitution and laws of the United States…"

176. As HHSC's executive commissioner, Ms. Young has general supervision and control over all matters relating to the health of the citizens of the State of Texas.[88]

177. The executive commissioner's office has rulemaking and enforcement authority over licensed medical facilities, immunization providers, and healthcare providers in Texas.

178. Subject to the oversight of the executive commissioner, the department[89] shall examine, investigate, enter, and inspect any public place or building as the department determines necessary for the discovery and suppression of disease and the enforcement of any health or sanitation law of this state.[90]

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[87] "I, _____, do solemnly swear (or affirm), that I will faithfully execute the duties of the office of _____ of the State of Texas, and will to the best of my ability preserve, protect, and defend the Constitution and laws of the United States and of this State, so help me God."
[88] Texas Health and Safety Code § 12.0001, *et seq.*
[89] Texas Department of State Health Services (DSHS)
[90] Texas Health and Safety Code § 12.0011

179. Before Texas willfully participated in the CDC COVID-19 Vaccination Program (see *infra*), Texas HHSC assured the federal HHS that Texas would not use its authority to place individuals under outside pressure to participate in biomedical or behavioral research activities (e.g., CDC COVID-19 Vaccination Program) that were federally funded or under the federal government's authority.[91]

180. To comply with its legal FWA obligations, HHSC promulgated rules under 26 Tex. Admin. Code §925.4(b), which appears in the Chapter titled "Research Involving Health and Human Services Commission Services," and states in part:[92]

> (b) HHSC's guiding principle for all research involving human subjects is the protection of the personal rights, safety, well-being, privacy, and dignity of the subjects to:
>
> (1) ensure the protection of human subjects involved in research, HHSC promulgates this chapter and adopts by reference 45 Code of Federal Regulation (CFR) Part 46, Subparts A, B, C, and D.
>
> (2) ensure ethical principles are maintained when research involving human subjects is conducted, HHSC adopts by reference "The Belmont Report: Ethical Principles and Guidelines for the Protection of Human Subjects of Research, Report of the National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research" (April 18, 1979).

181. In October of 2020, Ms. Young started planning Texas's COVID-19 vaccination program in conjunction with the CDC and prospectively agreed to perform a variety of duties including, but not limited to:

> (1) recruit medical facilities to help the state administer the federally owned EUA/PREP Act drugs,

[91] Texas HHSC is under FWA00028877

[92] COVID-19 drugs under 21 U.S.C.§360bbb-3 authorization require research activities subject to 45 CFR 46 regarding any person to whom the EUA investigational drugs are administered.

(2) educate the public of the differences between an EUA drug and a drug approved for general commercial marketing by the FDA,

(3) instruct recruited parties how to lawfully administer the EUA drugs,

(4) ensure private parties would provide potential recipients of the drugs with the drug fact insert sheet,

(5) ensure recruited parties were made aware of "any EUA" and instruct them how to administer the product lawfully under the EUA,

(6) instruct recruited parties how to perform the ministerial act of accepting an individual's option under the EUA statute

(7) adopt the Provider Agreement as state policy,

(8) require any recruited party to sign the Provider Agreement,

(9) monitor recruited parties involved in the state's CDC Program and enforce the provisions of the Provider Agreement.

182. Ms. Young had to ensure each party she recruited completed the Provider Agreement properly, knew of its requirements, and maintained the Provider Agreement on file for three years.

183. Therefore, Ms. Young was fully aware or should have been aware, that line 12(a) of the Provider Agreement placed a duty upon medical facilities as "Organizations" under the Provider Agreement to "comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine" (including informing an individual of their right to accept or refuse an EUA product.)

184. Once the Secretary issued EUAs, Ms. Young had assigned duties as the "emergency response stakeholder," including, but not limited to

(1) recruit vaccination providers,

(2) ensure the provider has the EUA letter,

> (3) instruct the provider how to administer the EUA product according to the law,
>
> (4) ensure the provider placed the drug Fact Sheet into the hands of any potential recipient,
>
> (5) ensure the provider performed their duties consistently with the EUA letter, EUA Statute, and Provider Agreement as specifically outlined in each EUA.

185. Therefore, Ms. Young had discretionary authority to determine which party would help the State administer the federally owned EUA/PREP Act drugs. However, she only had a ministerial duty to ensure those recruited parties honored the Equal Protection and Due Process rights of potential recipients of the federally owned EUA/PREP Act drugs.

186. Moreover, Ms. Young was responsible for ensuring that the recruited state actors (i.e., Organizations under the Provider Agreement and Vaccination Providers under each EUA) informed any potential recipient of the right to accept or refuse without consequence.

187. Accepting an individual's option to accept or refuse is a purely ministerial act.

188. The Fourteenth Amendment placed a constitutional duty upon Ms. Young to ensure her recruited parties treated individuals choosing to accept equal to individuals choosing to refuse.

189. Both options were created specifically for the benefit of individuals considering participating in the administration of the federally owned EUA/PREP Act drugs.

190. Ms. Young's office developed materials to help facilitate the State's COVID-19 Vaccination Program, including an informational card entitled, "How to Become a Texas COVID-19 Vaccine Provider,"[93] a slide program showing how to recruit private parties,[94] and registration guidance.[95]

[93] See Exhibit H, Texas "How To Become a Texas COVID-19 Vaccine Provider" Informational Card
[94] See Exhibit I, Texas Recruiting Slideshow
[95] See Exhibit J, Texas Registration Guide

191. Ms. Young's office built an online portal to inform recruited organizations what is required of them as a vaccination provider and how to enroll.[96]

192. The site specifically informed recruited organizations that:

> (1) "Only providers registered through this site can receive and administer COVID-19 vaccine in Texas,"
>
> (2) they "must register at Enroll Texas IZ dshs.texas.gov,"
>
> (3) they must "complete the Centers for Disease Control and Prevention (CDC) COVID-19 Vaccination Program Provider Agreement,"
>
> (4) they must "list the healthcare providers at that location that would be responsible for vaccination."

193. One will not discover the required promotional or educational materials educating the public, recruited organizations, or potential EUA drug recipients of the lawful rights of individuals to refuse the federal COVID-19 EUA property without consequence.

194. As of the filing of this complaint, Ms. Young has not developed any materials educating the public or licensed facilities or providers of the rights a person holds under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) to accept or refuse.

195. Ms. Young informed recruited organizations of the following requirements of vaccination providers:

> "All COVID-19 Vaccine Providers … must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine. Providers must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws."[97]

196. Therefore, there can be no doubt regarding the knowledge Ms. Young possessed that the State and the State Actor organizations she recruited to help administer the CDC Program

[96] See Exhibit D, Texas Vaccination Provider Enrollment
[97] See Exhibit D, Texas Vaccination Provider Enrollment

were under legal, contractual, and constitutional obligations to perform the ministerial function of accepting an individual's autonomous choice to accept or refuse under the EUA statute since all drugs under the CDC Program were federally owned EUA drugs.

197. Moreover, the State could not delegate the public function of administering the federal COVID-19 property to private-party state actors without delegating the State's Fourteenth Amendment obligations.

198. The drugs in question (e.g., Pfizer-BioNTech COVID-19 Vaccine, Moderna Vaccine, and other EUA drugs) are FDA-labeled as investigational and federally funded, and that specific classification requires strict adherence to the legally effective informed consent doctrine requiring the State and persons acting under color of law to ensure individuals are not under outside pressures to accept administration of the product.

199. Plaintiffs are aware of and can prove that, in April 2021, mere months after the State began participating in the CDC Program, vaccination providers under the CDC Program began breaching their duties under the Provider Agreement and federal laws when they advised individuals that they would, in fact, be penalized if they refused the administration of the federally owned COVID-19 EUA/PREP Act drugs administered under the State's authority.

200. Vaccination providers in Texas required employees, contractors, and volunteers to inject the CDC Program's COVID-19 EUA/PREP Act drugs into their bodies under threat of penalty, which was outside the scope of the vaccination providers' authority and in direct violation of the EUA Statute, Provider Agreement, PREP Act, State Policy, and the Fourteenth Amendment.

201. This fact is a pivotal moment in Texas because Ms. Young had a lawful duty to enforce the provisions of the State's CDC Program and had full authority by the powers of her office to enforce those provisions.

202. Ms. Young's nonfeasance directly led vaccination providers to destroy the careers of the hardest-working healthcare professionals in the state.

203. In silence, Ms. Young stood by and watched vaccination providers engage in unlawful and outrageous conduct, directly violating the Provider Agreement and the federal laws that the State of Texas affirmatively promised to monitor and enforce.

204. Moreover, Ms. Young was made aware of those transgressions by the numerous lawsuits filed by licensed healthcare practitioners specifically relating to the State Actors recruited by Ms. Young not performing the ministerial act of accepting an individual's lawful right to refuse the federal COVID-19 EUA/PREP Act product.

205. What followed was a tsunami of unlawful activity within the State by other vaccination providers who willfully ignored their legal obligations and potential recipients' federal and constitutional rights to accept or refuse free from outside pressures.

206. The moment Ms. Young committed nonfeasance, she established a state-enforced custom overriding the federal constitution and usurped from Congress its authority to prohibit the involuntary participation of individuals in the administration of the federal COVID-19 EUA/PREP Act drugs.

207. Ms. Young displayed "deliberate indifference" to the rights of healthcare workers licensed by the State (*Estelle v. Gamble*, 429 U.S. 97 (1976)).

208. A "special relationship" between Ms. Young, licensed healthcare facilities, and licensed healthcare workers existed because Ms. Young placed those workers in a position of danger from private party state actors by failing to educate and train the private party state actors (i.e., Organizations under the Provider Agreement) to protect the Constitutional and federal

statutory rights of potential recipients of the EUA/PREP Act drugs and then to enforce those provisions.[98]

209. Ms. Young established the State's COVID-19 vaccination program with licensed medical facilities, and her deliberate indifference to her duties allowed those licensed medical facilities to subject licensed healthcare workers to unwanted and unwarranted financial, emotional, and legal dangers.

210. The state-enforced custom deprived all healthcare workers in the State of their property interest to refuse under the EUA statute.

211. It is a State-enforced custom because individuals in Texas are currently under threat of penalty by the State to repay thousands of dollars in unemployment benefits for no other reason than choosing the option to refuse the federal COVID-19 EUA/PREP Act drug.

212. Hospitals, mayors, private employers, and public institutions jumped onto the state-enforced custom. They mandated that which Congress and State policy prohibited—non-consensual use of federally owned COVID-19 EUA/PREP Act drugs.

213. The Supreme Court noted in *Adickes v. S. H. Kress & Co*., 398 U.S. 144 (1970) that the "Petitioner will have established a claim under § 1983 for violation of her equal protection rights if she proves that she was refused service by respondent because of a state-enforced custom…" (emphasis added)

214. Moreover, the *Adickes* Court held, "Based upon the language of the statute['s] legislative history, and judicial decisions, the words 'under color of a . . . custom or usage, of [a]

[98] "The Constitution is a charter of negative liberties," Judge Posner nevertheless declared that "[i]f the state puts a man in a position of danger from private persons and then fails to protect him, it will not be heard to say that its role was merely passive; it is as much an active tortfeasor as if it had thrown him into a snake pit." This "position of danger" test operates exactly as a "special relationship" to expose the state to section 1983 liability for failure to act. *Bowers v. DeVito*, 686 F.2d 616 (7th Cir. 1982); See also *Jensen v. Conrad*, 747 F.2d 185 (4th Cir. 1984)

State,' in § 1983, mean that the 'custom or usage' must have the force of law by virtue of the persistent practices of state officials." *Id.*, at 162-169.

215. Whereas the plaintiff in *Adickes* was refused service, Plaintiffs herein were refused employment for no other reason than an illegal state-enforced custom established by Ms. Young's deliberate indifference to her lawful duties.

216. There can be no doubt that since the beginning of the Texas CDC Program, the "custom" has been "persistent practices of state officials" because no state official enforces the right of individuals to refuse any EUA drug and/or a product covered under the PREP Act.

217. Moreover, the State's recruited vaccination providers violated their lawful obligations, with Ms. Young's approval, and used the State's at-will employment laws to retaliate against those individuals. As set forth above, the PREP Act expressly preempted those laws when used for that purpose.

218. The *Adickes* Court noted, "[T]he chief complaint is not that the laws of the State are unequal, but that, even where the laws are just and equal on their face, yet, by a systematic maladministration of them, or a neglect or refusal to enforce their provisions, a portion of the people are denied equal protection under them."

219. The State agreed to treat the option to accept equal to the option to refuse, and yet, by Ms. Young's refusal to enforce the provisions of the Fourteenth Amendment, CDC Program, Provider Agreement, its FWA, and the EUA statute, she established a custom where "a portion of the people are denied equal protection under" the federal constitution and their statutory rights.

220. Ms. Young's "maladministration" of the CDC Program established a second-class citizenry where individuals who refused were penalized, but those who chose to accept were not.

221. Texas received over $959 per capita in COVID-19 funding for persons participating in the CDC Program, or an estimated $27b.[99]

222. These financial incentives caused Ms. Young to violate the duties of her office at the expense of destroying the Plaintiffs' financial lives for the ill-gotten gain of the State.

223. The EUA letters informed Ms. Young that as the "emergency response stakeholder" and "jurisdictional authority" over the CDC Program, her duties were to "identify vaccination sites to receive authorized Pfizer-BioNTech COVID-19 Vaccine **and ensure its** distribution and **administration**, **consistent** with the terms of this letter and CDC's COVID-19 Vaccination Program" (emphasis added). Ms. Young breached those duties.

224. Ms. Young's recruited parties did not consistently administer the CDC Program and its applicable federal laws.

225. Untold thousands of individuals had their financial lives destroyed by Ms. Young's willful neglect of duty.

226. Moreover, the state-enforced custom established by Ms. Young's outrageous conduct continues unabated by hospitals, educational facilities, and other entities refusing to acknowledge or respect an individual's right to refuse the administration of a drug or device authorized under the EUA Statute and/or covered under the PREP Act.

227. "A person, 'subjects' another to the deprivation of a constitutional right, within the meaning of section 1983, if he does an affirmative act, participates in another's affirmative acts, or omits to perform an act which he is legally required to do that causes the deprivation of which complaint is made." *Johnson v. Duffy*, 588 F.2d 740, 743 (9th Cir. 1978) citing *Sims v. Adams* (5th Cir. 1976)

[99] See Exhibit K, Peter G. Peterson Foundation Website Screenshot, Accessed 2/20/2024

228. Ms. Young failed to "monitor" recruited parties who were "clothed with the authority of state law" and enforce the right of potential recipients to refuse an EUA/PREP Act drug.

229. Ms. Young misused her power by developing a state-enforced custom having the force of law greater than the State's legislature, federal constitution, and the U.S. Congress. [100]

230. The State, having a duty to prospectively obtain the legally effective informed consent of individuals on behalf of the federal government, could not delegate its public function of administering the federal EUA/PREP Act drugs without also delegating that duty.

231. The custom is directly attributed to Ms. Young's willful failure to perform the duties of her office, instruct the vaccination providers of their lawful obligations, ensure that she established conditions informing individuals of their right to accept or refuse the federally owned COVID-19 EUA/PREP Act drugs, to use the powers of her office to protect individuals' autonomous choice by "monitor[ing] activities at the local level **to ensure** the COVID-19 Vaccination Program is implemented throughout the jurisdiction in adherence with federal guidance and requirements," and to "ensure its distribution and administration [is] consistent with the terms of [the EUA] letter and CDC's COVID-19 Vaccination Program" because her office and "Organizations" she recruited to participate in the federal government's CDC Program promised to abide by "any EUA that covers COVID-19 Vaccine."

232. Ms. Young's failure to perform her duties encouraged Shriners Hospitals for Children to violate its known legal requirements, knowing that Ms. Young's office would not

[100] Misuse of power, possessed by virtue of state law and made possible only because the wrongdoer is clothed with the authority of state law, is action taken "under color of" state law. *United States v. Classic*, 313 U.S. 299 (1941), citing *Ex parte Virginia*, 100 U. S. 339, 100 U. S. 346; *Home Telephone & Telegraph Co. v. Los Angeles*, 227 U. S. 278, 227 U. S. 287, *et seq*.; *Hague v. CIO*, 307 U. S. 496, 307 U. S. 507, 307 U. S. 519; cf. 101 F.2d 774, 790

penalize Shriners for destroying the financial lives of their healthcare workers whose only crime was exercising their right under the EUA statute to refuse without penalty.

233. Ms. Young failed to instruct recruited parties on the proper and lawful administration of the CDC Program.

234. Ms. Young failed to monitor and enforce the provisions of the Provider Agreement among her recruited parties.

235. Ms. Young's deliberate indifference to the legal requirements under the CDC Program and applicable laws deprived Plaintiffs of their Fourteenth Amendment right to Equal Protection.

236. Ms. Young's deliberate indifference to the legal requirements under the CDC Program and applicable laws deprived Plaintiffs of their fundamental right to refuse administration of an EUA product (investigational new drug) into their bodies.

237. Ms. Young's deliberate indifference to the legal requirements under the PREP Act deprived Plaintiffs of their fundamental right to refuse the administration of a PREP Act-covered countermeasure into their bodies.

238. Ms. Young's deliberate indifference to the legal requirements under the CDC Program and applicable laws violated Plaintiffs' privacy rights when they were required to inform the recruited parties whether Plaintiffs had been administered the unlicensed drugs.

239. Ms. Young's deliberate indifference to the legal requirements under the CDC Program, PREP Act, and applicable laws established an unconstitutional condition where state-licensed healthcare workers were provided a choice between surrendering their due process (fundamental), equal protection, and privacy rights or remaining employed within the healthcare industry under the State's licensing requirement.

240. Ms. Young's willful betrayal of her oath of office created an unspoken State license requirement among the State's licensed medical facilities that a healthcare worker must surrender their Equal Protection, Due Process, and statutory rights and inject the federal COVID-19 property into their bodies as a condition to work in the State's healthcare industry.

241. Ms. Young's deliberate indifference to her legal duties is atrocious and outrageous, and never before in the history of the State of Texas has one individual been directly involved in the complete deprivation of constitutional rights of an entire labor force.

242. Plaintiffs lost the use of their state-issued personal property (licenses to work as a healthcare provider), employment, health insurance, homes, autos, education, retirement savings, emotional well-being, financial well-being, and equality before the law.

243. Ms. Young achieved by reckless and wanton disregard for the duties of her office which she could not achieve by the judicial or legislative branches of the State government—penalizing individuals refusing the federally owned investigational drugs.

B. Defendants, Shriners PolicyMakers

244. Shriners Hospitals for Children is a network of not-for-profit medical facilities incorporated in Colorado.

245. Shriners is licensed by the State of Texas to conduct business as a hospital.

246. At all times pertinent, Plaintiffs were employed by Shriners Hospitals for Children, located at 815 Market Street, Galveston, Texas, and were subject to the authority of Shriners PolicyMakers.

247. At all times pertinent, Shriners PolicyMakers were acting under color of law as a State Actor under the State's CDC Program and HHSC's State-enforced custom when engaging in the challenged conduct.

248. Shriners PolicyMakers signed the CDC COVID-19 Vaccination Agreement.

249. Shriners PolicyMakers operate under FWA00025698 whenever they involve humans with federally funded investigational new drugs.

250. Shriners Hospitals for Children is an "organization" under the federal Provider Agreement and subject to its terms and conditions.

251. Shriners Hospitals for Children is a "vaccination provider" under any EUA.

252. On September 14, 2021, William S. Bailey, Imperial Potentate of Shriners International, Jerry G. Gantt, Chairman of the Board of Trustees of Shriners Hospitals for Children, and John P. McCabe, Executive Vice President of Shriners Hospitals for Children, issued and signed an unlawful directive to "all Shriners Hospitals for Children Employees and Contract Staff" regarding a new company policy titled, "COVID-19 Vaccine Policy"[101] ("Policy").

253. The memo stated in part:

A. "Last week the Joint Boards approved a policy requiring nearly everyone in our organization to get fully vaccinated against COVID-19."

B. "Everyone at every Shriners Hospitals for Children location in the United States and Canada, including Headquarters, must comply with the policy. All employees, contract employees, vendors, and volunteers who have a need to enter a building owned and/or operated by Shriners Hospitals for Children are required to be fully vaccinated against the COVID-19 virus."

C. "Everyone will need their first shot of the vaccination series by October 11, and must be fully vaccinated or have an appropriate exemption approved by December 6."

D. "It is a requirement of employment with Shriners Hospitals for Children that all eligible persons be fully vaccinated. Those few who receive exemptions will be required to follow strict protocols that are anticipated to include masking with a special N95 mask and weekly COVID testing."

[101] See Exhibit C, Shriners Policy

E. "The requirements for medical or religious exemptions are specific. Talk to your local Human Resources team about your personal situation. To apply for an exemption, you must submit supporting documentation as well as a completed request form. All exemption requests will be vetted by a team including corporate leadership."

254. At all times pertinent, the only drugs available to Plaintiffs for compliance with Shriners PolicyMakers Policy were the federally owned COVID-19 drugs classified as investigational and subject to the Provider Agreement, State COVID-19 Program, the EUA statute, the PREP Act, the State's FWA, and Shriners' FWA.

255. Black letter law dictates that the use of an EUA/PREP Act investigational drug can never be mandated.[102]

256. The federal government prohibited Shriners PolicyMakers under its FWA and IRB agreements to mandate an investigational drug even if Shriners PolicyMakers were to claim that using the drug is voluntary because the listed penalties for noncompliance result in outside pressure being placed on the potential recipient, in violation of the EUA statute, 45 C.F.R. § 46.116, and the Belmont Report

257. The Policy, as applied to the exclusive use of the federal COVID-19 property, was *ultra vires*, unlawful, and the direct cause of action of Plaintiffs'' injuries.

258. Shriners PolicyMakers enacted the Policy, executed the Policy, and required its management to enforce the Policy. Therefore they are persons that "'subject' another to the deprivation of a constitutional right, within the meaning of section 1983, if he does an affirmative act, participates in another's affirmative acts, or omits to perform an act which he is legally required to do that causes the deprivation of which complaint is made." *Johnson v. Duffy*, 588 F.2d 740, 743 (9th Cir. 1978) citing *Sims v. Adams* (5th Cir. 1976)

[102] 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III); 42 U.S.C. § 247d-6d and 42 U.S.C. § 247d-6e

259. Plaintiffs were subjected to the use of the federally owned COVID-19 EUA/PREP Act drugs under threat of penalty, and when they refused, Shriners PolicyMakers gaslit Plaintiffs, who were educated, informed, and instructed by Shriners PolicyMakers for decades that they must obtain an individual's legally effective informed consent anytime they administer an investigational drug to an individual.

260. Plaintiffs knew their right to refuse the federally owned EUA/PREP Act drugs. Still, Shriners PolicyMakers acted with moral turpitude, pretended that the laws did not apply to Plaintiffs, and deprived them of their statutory and constitutional rights.

261. Shriners PolicyMakers are legally sophisticated in laws applicable to the investigational new drug classification. They know or should know, they are legally restricted from applying outside pressure on Plaintiffs to participate in federally funded research projects, much less sanctioning them for refusing to participate.

262. Shriners PolicyMakers oversee a multi-million-dollar research program utilizing INDs, which require strict adherence to 45 C.F.R. Part 46, the Belmont Report, and the EUA statute.[103]

263. Moreover, Shriners PolicyMakers routinely administer investigational new drugs authorized under various sections under 21 U.S.C. §360bbb *et. seq*., and know, or should know, that an individual has the explicit right to refuse an investigational drug without consequence.

264. At no time during Shriners' history has a federal law allowed Shriners PolicyMakers to mandate compulsory use of an unlicensed investigational drug as a condition of employment.

[103] https://www.shrinerschildrens.org/en/research-and-expertise/research-at-shriners-childrens

265. To the contrary, such a requirement violates Shriners PolicyMakers' agreements with the federal government and federal law.

266. Shriners PolicyMakers cannot produce a constitution, statute, regulation, federal agreement, or other legal authority providing it the right to condition employment upon the use of an EUA/PREP Act investigational new drug or device.

267. Therefore, Shriners PolicyMakers misrepresented their authority when amending the conditions of authorization for each EUA, as established by the Secretary, requiring nonconsensual use of an EUA/PREP Act drug by Plaintiffs as a condition of employment.

268. Shriners PolicyMakers usurped from Congress its authority to prohibit the involuntary use of EUA/PREP Act drugs or devices when issuing and enforcing its illegal Policy as applied.

269. Moreover, the federal-government-owned COVID-19 drugs were labeled as investigational, but Shriners PolicyMakers promoted the drugs as if they were FDA-licensed with a legal indication.

270. Shriners' PolicyMakers' intent was to cause Plaintiffs to inject the investigational new drugs into their bodies under the false belief that the drugs were something other than what was on their label.

271. Such an act is a violation of federal misbranding laws and the "Conditions Related to Printed Matter, Advertising, and Promotion" outlined in each EUA stating that "All descriptive printed matter, advertising, and promotional material, relating to the use of the Pfizer-BioNTech COVID-19 Vaccine shall be consistent with the authorized labeling" and such content will "clearly and conspicuously shall state that: This product has not been approved or licensed by FDA, but has been authorized for emergency use by FDA." (21 U.S.C. § 331)

272. That specific EUA condition aims to ensure federal drug labeling laws are not violated.

273. None of Shriners PolicyMakers' materials relating to any EUA drug informed any individual that the FDA did not approve the drugs nor that the individual had the explicit right to refuse the product without non-medical consequence.

274. Shriners PolicyMakers committed fraud by concealing Plaintiffs' legal rights, which were identified to Shriners PolicyMakers in the Provider Agreement, the State Agreement, EUA letters, and 21 U.S.C. §360bbb-3 and its FWA.

275. Shriners PolicyMakers had a duty to prospectively obtain Plaintiffs' legally effective informed consent, which they intentionally failed to do.

276. Obtaining this form of consent requires establishing a legally approved environment, ensuring the Plaintiffs were not under a sanction for refusing.

277. Shriners PolicyMakers, acting under color of state law, penalized Plaintiffs because they chose one of the two legally protected options under the EUA statute, thus violating Plaintiffs' legally effective informed consent rights.

278. Shriners PolicyMakers were legally preempted by the EUA statute via the Supremacy Clause and expressly preempted by the express terms of the PREP Act from establishing conditions interfering with the Plaintiffs' chosen option.

279. Shriners Hospitals for Children was the "Organization" under the Provider Agreement bound to line 12(a) under the Provider Agreement.

280. The Provider Agreement required Shriners Hospitals for Children to adhere to the EUA statute and perform the ministerial act of accepting an individual's choice irrespective of the individual's relationship with Shriners (e.g., employee, volunteer, contractor, patient).

281. Shriners PolicyMakers were "vaccination providers" under the respective EUA letters.[104]

282. The EUA letters established that vaccination providers had a ministerial duty to "administer the vaccine in accordance with the authorization and will participate and comply with the terms and training required by CDC's COVID-19 Vaccination Program."

283. The terms of the CDC Program included the ministerial duty of informing individuals of their right to accept or refuse the product without a non-medical consequence and then accepting the chosen option.

284. Thus, Shriners PolicyMakers acted under color of law when they informed Plaintiffs that they would be penalized if they chose to refuse.

285. 42 U.S.C. § 1983 states:

> Every person who, under color of any statute, ordinance, regulation, custom, or usage, of any State or Territory or the District of Columbia, subjects, or causes to be subjected, any citizen of the United States or other person within the jurisdiction thereof to the deprivation of any rights, privileges, or immunities secured by the Constitution and laws, shall be liable to the party injured in an action at law, suit in equity, or other proper proceeding for redress…

286. Very recently, the U.S. Supreme Court reiterated that a federal statute conferring a right onto an individual in unambiguous language is enforceable under 42 U.S.C. §1983.[105],[106]

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[104] See Exhibit B, p. 10, Letter R

[105] *Maine v. Thiboutot*, 448 U.S. 1 (1980), the court held that "Even were the language ambiguous, however, any doubt as to its meaning has been resolved by our several cases suggesting, explicitly or implicitly, that the §1983 remedy broadly encompasses violations of federal statutory as well as constitutional law." See also, *Health and Hospital Corporation of Marion Cty. V. Talevski*, 599 U.S. 166 (2023).

[106] *Gonzaga* sets forth our established method for ascertaining unambiguous conferral. Courts must employ traditional tools of statutory construction to assess whether Congress has "unambiguously conferred" "individual rights upon a class of beneficiaries" to which the plaintiff belongs. [citations omitted] Notably, it must be determined that "Congress intended to create a federal right" *for* the identified class, not merely that the plaintiffs fall "within the general zone of interest that the statute is intended to protect" *Gonzaga*, 536 U.S., at 283. This paradigm respects Congress's primacy in this arena and thus vindicates the separation of powers. *Id*., at 286. Id., at 284, 287. Conversely, we have rejected § 1983 enforceability where the statutory provision "contain[ed] no rights-creating language"; had "an aggregate, not individual, focus"; and serve[d] primarily to direct the [Federal Government's] distribution of public

287. The *Talevski* Court[107] held that language in the Federal Nursing Home Reform Act that a nursing home resident had the "right to be free from . . . any physical or chemical restraints" and had the right to advanced notice of discharge created a right enforceable by § 1983.

288. Similarly, in the case at bar, the language in the EUA statute that "individuals to whom the product is administered are [to be] informed…of the option to accept or refuse administration of the product" creates a right enforceable by § 1983.

289. This EUA statutory language also speaks in terms of "individual rights upon a class of beneficiaries" to which the plaintiff belongs.

290. The statute actually uses the word "individuals" when describing to whom the right is conferred.

291. The class of beneficiaries are those contemplating "administration of the product."

292. Congress intended to create a federal right for the identified class by describing the right the way it did.

293. The State of Texas was not required to participate in the CDC Program, but it exercised its right to do so and willfully consented to the terms of the Provider Agreement.

294. Unlike other federal programs, such as Medicare, where private party hospitals bill their patients for the medical services provided, the CDC Program strictly prohibits Organizations from charging for the COVID-19 EUA/PREP Act drugs.

295. Unlike other federal programs, such as Medicare, where private party hospitals can be sued by their patients for medical malpractice, the CDC Program provides statutory immunity to Organizations that administer the COVID-19 EUA/PREP Act drugs.

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funds." *Id*., at 290 "We have held that the *Gonzaga* test is satisfied where the provision in question is "phrased in terms of the persons benefited" and contains "rights-creating," individual-centric language with an "unmistakable focus on the benefited class." *Health and Hospital Corporation of Marion Cty. V. Talevski*, 599 U.S. 166 (2023).

[107] *Health and Hospital Corporation of Marion Cty. V. Talevski*, 599 U.S. 166 (2023).

296. Unlike other federal programs where hospitals have discretionary authority to carry out their services, hospitals that agreed to the CDC Program were not provided deference to how they conduct the Program.

297. The duties and responsibilities of the State and Organizations involved in the CDC Program were detailed, explicit, strict, and tightly controlled by State authority.

298. First, the State was exclusively and legally responsible for administering the federally owned COVID-19 EUA/PREP Act drugs.

299. Second, the State agreed to very specific requirements about how it and the parties it recruited could operate under the CDC Program.

300. Third, the State was bound to conduct research activities on behalf of the federal government.

301. Fourth, the State was bound to obtain individuals' legally effective informed consent before they were offered the EUA/PREP Act drugs.

302. Lastly, the State was bound to ensure that Organizations under the Provider Agreement were made aware that the drugs were not approved or licensed for any legal indication but only authorized for emergency use.

303. Individuals who consent to use an EUA product or PREP Act countermeasure:

A. forfeit civil litigation rights resulting from injuries;[108]

B. allow their private identifiable information to be collected and used for a variety of purposes by unknown persons;[109]

C. allow their involvement with the EUA product to be cataloged by various persons for unknown purposes,

[108] PREP Act forfeits all civil actions for damages in most situations.

[109] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Program requires manufacturers and/or emergency stakeholders to obtain private identifiable information.

D. allow the data collected about their adverse events to be utilized by researchers for unknown purposes and for eternity,[110]

E. assume greater risks to their safety, health, and legal rights.[111]

304. The FDA specifically stated that Pfizer's two drugs (COMIRNATY® and Pfizer-BioNTech COVID-19 Vaccine) were legally distinct in its August 23, 2021, EUA letter to Pfizer.[112]

305. Therefore, the legal distinctions between Pfizer's two drugs are its labeling and the laws applicable to the label.

306. Drugs are regulated according to their labeling and never according to their formulations.

307. Laws regulating investigational drugs are entirely different from those regulating a FDA-licensed drug.

308. The laws applicable to individuals participating in activities under the EUA statute and PREP Act have severe legal, financial, and potentially physical injury or damage, which is why Congress required the State to obtain the legally effective informed consent of individuals anytime they involve them in federally funded research projects (i.e., CDC Program) or investigational products (i.e., Pfizer-BioNTech COVID-19 Vaccine).

309. The State required Shriners PolicyMakers to conduct research activities on its behalf as outlined in each EUA, the Provider Agreement, and the State Policy.[113]

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[110] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Program requires manufacturers and/or emergency stakeholders to monitor, report and study a variety of adverse reactions to EUA products.

[111] 21 U.S.C. §360bbb-3 requires potential recipients to be made aware of the risks, alternatives, and the fact that the product is only authorized by the Secretary under emergency conditions. These elements provide potential recipients with the required information to make a quality and legally effective decision to consent. Therefore, consent means the individual agrees to assume more than minimal risk as defined in 21 CFR 50.3(k).

[112] See Exhibit B, Footnote 8

[113] See Exhibit D, Texas Vaccination Provider Enrollment

310. The State required Shriners to adhere to ministerial duties under the EUA statute of informing potential recipients of the COVID-19 EUA/PREP Act drugs that they have the right to accept or refuse without penalty.

311. The State needed healthcare facilities, such as Shriners Hospitals for Children, to help achieve its goal of administering the federally owned COVID-19 EUA/PREP Act property, which in turn helped increase the revenue to the State from the CDC Program.

312. The State was explicit in who, when, where, and to whom the product could be administered and the conditions under which the administrative activities could occur.

313. The Vaccine Information Fact Sheet for Recipients and Caregivers provides the following information to be given to a potential recipient who refuses the injection, as follows:

> Under the EUA, it is your choice to receive or not receive any of these vaccines. Should you decide not to receive any of these vaccines, it will not change your standard medical care.[114]

314. The State required the private party to open its doors to the public and prevent the private party from charging any individual for the product, ancillary supplies, or office visit, nor could the private party turn away a person who could not pay for the visit.

315. However, after requiring strict adherence to its policies, the State established a State-enforced custom allowing Shriners Hospitals for Children to penalize individuals who refused the federally owned COVID-19 EUA/PREP Act drugs.

C. Under Color of Law

316. The State cannot delegate the public function of administering the federally owned COVID-19 EUA/PREP Act drugs without also delegating its legal and Constitutional obligations.

[114] See Exhibit L, Fact Sheet, p. 6.

317. Therefore, Shriners PolicyMakers, acting on behalf of the State, owed statutory duties under the EUA statute, the PREP Act, 10 U.S.C. § 980, 45 CFR Part 46, the Belmont Report, and the Provider Agreement and the State's obligations outlined under the Fourteenth Amendment.

318. The Supreme Court utilizes several State Action Tests to ascertain if a private party is acting under the color of law.

319. The **Public Function Test,** at first blush, would appear inapplicable, but no person has the authority to administer the federally owned COVID-19 EUA/PREP Act drugs outside of the CDC Program and without State authority or the State's IIS.

320. The State, not Shriners PolicyMakers, has exclusive and complete authority over the federal emergency program operating within the State.

321. The public function test is met by:

> (1) Shriners exercising powers exclusively reserved for the federal government and Texas,[115]
>
> (2) the powers are part of Texas's exclusive right to administer the CDC Program within the State of Texas[116] and
>
> (3) the powers are such that the State itself is obligated to perform because Governor Abbott voluntarily agreed to perform the State's role under the CDC Program.[117]

322. The U.S. Supreme Court held in *Jackson v. Metropolitan Edison Co*., 419 U.S. 345 (1974), "We have, of course, found state action present in the exercise by a private entity of powers traditionally exclusively reserved to the State."

[115] *Jackson v. Metropolitan Edison Co*., 419 U.S. at 352
[116] *Flagg Bros., Inc. v. Brooks*, 436 U.S. at 160. See also *Jackson v. Metropolitan Edison Co*., 419 U.S. at 353
[117] *Jackson v. Metropolitan Edison Co*., 419 U.S. at 353

323. The regulation, distribution, and administration of emergency use investigational drugs that are not licensed for any indication is "traditionally exclusively reserved to the State" in coordination with the federal government.

324. No private party is legally allowed to regulate, distribute, or administer emergency-use investigational drugs that are not licensed for any indication of its own volition.

325. **The symbiotic relationship and or Joint Action test,**[118] which requires substantial coordination and integration between the private entity and the government, is met because:

(1) Texas was required to obtain Plaintiffs' legally effective informed consent, and they relied upon the Shriners PolicyMakers for that function;

(2) Texas owed Fourteenth Amendment obligations to Plaintiffs, and Texas delegated those obligations to Shriners PolicyMakers[119];

(3) Texas and Shriners PolicyMakers were required to conduct ongoing research activities on behalf of the federal government;

(4) Texas did not give discretion to Shriners PolicyMakers on how they could conduct the activities of the COVID-19 Vaccination Program[120];

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[118] *Brunette v. Humane Society of Ventura County*, 294 F.3d 1205 (9th Cir. 2002): "*Burton* (*Burton v. Wilmington Pkg. Auth*, 365 U.S. 715, 81 S. Ct. 856 (1961)) teaches that substantial coordination and integration between the private entity and the government are the essence of a symbiotic relationship. Often significant financial integration indicates a symbiotic relationship. *See Rendell-Baker v Kohn*, 457 U.S. at 842-43, 102 S.Ct. 2764; *Vincent v. Trend W. Tech. Corp*., 828 F.2d 563, 569 (9th Cir. 1987). For example, if a private entity, like the restaurant in *Burton*, confers significant financial benefits indispensable to the government's "financial success," then a symbiotic relationship may exist. *Vincent*, 828 F.2d at 569. A symbiotic relationship may also arise by virtue of the government's exercise of plenary control over the private party's actions. *See Dobyns v. E-Systems, Inc*., 667 F.2d 1219, 1226-27 (5th Cir. 1982) (finding symbiotic relationship where the government-controlled a private peacekeeping force engaged in a government-directed field mission in the Sinai Peninsula).

[119] In *Giron v. Corrections Corp. of America*, 14 F. Supp. 2d 1245 (D.N.M. 1998), the court stated, "If a state government must satisfy certain constitutional obligations when carrying out its functions, it cannot avoid those obligations and deprive individuals of their constitutionally protected rights by delegating governmental functions to the private sector. See *Terry v. Adams*, 345 U.S. 461, 73 S. Ct. 809, 97 L. Ed. 1152 (1953). The delegation of the function must carry with it a delegation of constitutional responsibilities."

[120] As the court held in *Modaber v. Culpeper Memorial Hospital, Inc*., 674 F.2d 1023 (4th Cir. 1982): "we must inquire 'whether there is a sufficiently close nexus between the State and the challenged action of the regulated entity that the action of the latter may fairly be treated as that of the State itself." *Jackson v. Metropolitan Edison Co*., 419 U.S. 345, 351, 95 S.Ct. 449, 453, 42 L.Ed.2d 477 (1974); accord, *Flagg Brothers, Inc. v. Brooks*, 436 U.S. 149, 157, 98 S.Ct. 1729, 1733, 56 L.Ed.2d 185 (1978). In holding that a privately-owned utility's termination of service is not "state action", the Court in *Jackson* makes it clear that state involvement without state responsibility cannot establish this nexus. See 419 U.S. 358, 95 S.Ct. 457. A state becomes responsible for a private party's act if the private party acts

(5) Texas relied upon State-authorized State Actors to achieve its goal of administering the federally owned COVID-19 EUA/PREP Act drugs[121]

(6) Texas relied upon State Actors to perform its ministerial duty of accepting an individual's chosen option under the EUA statute.

326. Shriners Hospitals for Children was physically and financially an integral part of the State's COVID-19 program, participated in the State's CDC Program using federal funds, offered the product to the public, including to state-licensed healthcare workers, and was a joint participant in the operation of the CDC Program.

327. When the State agreed to administer the federally owned COVID-19 EUA/PREP Act drugs, the Fourteenth Amendment had to be adhered to "as though they were binding covenants written into the agreement itself."[122]

328. Congress determined that once enrolled, persons involved in any administrative aspect of an EUA program would only have ministerial duties to inform an individual of their option to accept or refuse.

329. The Supreme Court held that "to act 'under color of' state law for § 1983 purposes does not require that the defendant be an officer of the State. It is enough that he is a willful participant in joint action with the State or its agents. Private persons, jointly engaged with state officials in the challenged action, are acting 'under color' of law for purposes of § 1983 actions,"[123]

(1) in an exclusively state capacity, (2) for the state's direct benefit, or (3) at the state's specific behest. It acts in an exclusively state capacity when it "exercises powers traditionally exclusively reserved to the state[,]" 419 U.S. 352, 95 S.Ct. 454; for the state's direct benefit when it shares the rewards and responsibilities of a private venture with the state, see id., 357-58, 95 S.Ct. 456-57, *Burton v. Wilmington Parking Authority*, 365 U.S. 715, 723-24, 81 S.Ct. 856, 860-61, 6 L.Ed.2d 45 (1961); and at the state's specific behest when it does a particular act which the state has directed or encouraged."

[121] *Burton v. Wilmington Parking Authority*, 365 U.S. 715 (1961)

[122] *Burton v. Wilmington Parking Authority*, 365 U.S. 715 (1961)

[123] *Dennis v. Sparks*, 449 U.S. 24 (1980)

and "if a private actor is functioning as the government, that private actor becomes the state for purposes of state action."[124]

330. **The Customs Test.** The Supreme Court held that the "Petitioner will have established a claim under §1983 for violation of her equal protection rights if she proves that she was refused service by respondent because of a state-enforced custom…"[125]

331. The State and Shriners PolicyMakers were both[126] under strict adherence to comply with the Provider Agreement, their respective FWAs, the EUA statute, and each EUA letter.

332. The State was required to enforce an individual's right to refuse the federally owned COVID-19 EUA/PREP Act drugs without consequence. Shriners PolicyMakers were required to provide a Fact Sheet and accept the potential recipient's chosen option.

333. The EUA statute and the Provider Agreement explicitly prohibits Shriners PolicyMakers from penalizing Plaintiffs for exercising their right to refuse an EUA/PREP Act drug, so when Shriners PolicyMakers did so, they acted on a State-enforced custom established by Ms. Young through maladministration of the CDC Program and refusal to enforce the laws regulating that Program.

334. Shriners PolicyMakers were "clothed with the authority of state law" to participate in the CDC Program. They were authorized by State-enforced custom to "misuse" that power and deprive Plaintiffs of their statutory and constitutional rights.

335. Penalizing Plaintiffs for exercising their right to refuse federally owned COVID-19 EUA/PREP Act drugs is an act that "the state has directed or encouraged."[127]

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[124] *Terry v. Adams*, 345 U.S. 461, 469-70, 73 S. Ct. 809, 97 L. Ed. 1152 (1953); *See Jackson v. Metropolitan Edison Co.*, 419 U.S. at 353, 95 S. Ct. 449.
[125] *Adickes v. S. H. Kress & Co*., 398 U.S. 144 (1970) (Emphasis added)
[126] "This program is a part of collaboration under the relevant state, local, or territorial immunization's cooperative agreement with CDC." See Exhibit A, Provider Agreement.
[127] *Modaber v. Culpeper Memorial Hospital, Inc*., 674 F.2d 1023 (4th Cir. 1982)

336. The State reinforced this custom by denying healthcare workers unemployment benefits when they were terminated for no other reason than refusing an EUA/PREP Act drug.

337. There can be no reasonable argument that "The State has so far insinuated itself into a position of interdependence…that it must be recognized as a joint participant in the challenged activity."[128]

338. The U.S. Supreme Court held private party state action "may occur if the State creates the legal framework governing the conduct, e.g., *North Georgia Finishing, Inc. v. Di-Chem, Inc*., 419 U.S. 601 (1975); if it delegates its authority to the private actor, e.g., *West v. Atkins*, 487 U.S. 42 (1988); or sometimes if it knowingly accepts the benefits derived from unconstitutional behavior, e.g., *Burton v. Wilmington Parking Authority*." *National Collegiate Athletic Ass'n v. Tarkanian*, 488 U.S. 179, 192 (1988)

339. In the instant action, the State knowingly benefited (financially) from the unconstitutional conduct of the licensed healthcare facilities to which it delegated its authority, and it accepted that benefit to the tune of $27 billion dollars.

340. As the Supreme Court has stated many times, "a State normally can be held responsible for a private decision only when it has exercised coercive power or has provided such significant encouragement, either overt or covert, that the choice must in law be deemed to be that of the State."[129]

341. The State agreed to perform the ministerial act of accepting a person's chosen option under the EUA statute and then delegated its function to untold hundreds, if not thousands, of private actors but acted with deliberate indifference to its legal obligation by allowing those

[128] *Burton v. Wilmington Pkg. Auth*, 365 U.S. 715, 81 S. Ct. 856 (1961)
[129] *Flagg Brothers, Inc. v. Brooks*, 436 U. S. 149 (1978); *Jackson v. Metropolitan Edison Co*., 419 U. S. 345 (1974); *Moose Lodge No. 107 v. Irvis*, 407 U. S. 163 (1972); and *Adickes v. S. H. Kress Co*., 398 U. S. 144 (1970)

private parties to violate federal law, the CDC Program, and the State's Fourteenth Amendment obligations.

342. Therefore, the state actors (i.e., vaccination providers under the CDC Program) were indeed acting under "significant encouragement" that was "covert" by the State. Thus, the actions of the private parties are deemed to be that of the State itself.

343. At all times pertinent, Shriners PolicyMakers were acting under color of law because the State knew that its recruited parties were violating requirements under federal law, a federal program, and the federal constitution, and the State allowed that unconstitutional behavior to continue because it derived benefits from the unlawful activity.

344. Moreover, the State supported the unlawful activity by denying the victims of the unconstitutional conduct unemployment benefits when they exercised their option to refuse.

345. Shriners PolicyMakers and the State engaged in the joint conduct of violating the terms and conditions of the CDC Program, their respective FWAs, and ministerial duties of the EUA statute.

346. Shriners PolicyMakers did not have the authority to amend the Scope of Authorization for each EUA that their Policy effectuated.

347. Shriners PolicyMakers did not have the authority to condition employment upon Plaintiffs accepting the administration of federally funded investigational drugs.

348. Shriners PolicyMakers deprived Plaintiffs of their Equal Protection rights when treating Plaintiffs' chosen option differently than individuals choosing a different but equal option.

349. Shriners Policymakers deprived Plaintiffs of their fundamental right to refuse administration of an EUA product into their bodies.

350. Shriners PolicyMakers deprived Plaintiffs of their fundamental right to refuse administration of a PREP Act, covered countermeasure into their bodies.

351. Shriners PolicyMakers violated Plaintiffs' Privacy Rights when requiring Plaintiffs to involuntarily surrender their private identifiable health information to unknown persons for unknown reasons for eternity.

352. Additionally, Shriners PolicyMakers violated Plaintiffs' Privacy Rights because Plaintiffs' biomedical data could be utilized for research purposes specifically because the CDC Program was under 45 C.F.R. Part 46.

353. Moreover, Shriners PolicyMakers required Plaintiffs to inform Shriners if Plaintiffs were administered an investigational drug, which is a purely private matter and not required by any law.

354. An investigational drug is not a licensed vaccine, and thus, the product could not come under any vaccine mandate.

355. Shriners PolicyMakers devoted decades to educating its staff on the rights of individuals when its organization involves an individual with an investigational new drug, EUA product, or products under their FWA agreement.

356. However, when it came time to protect the rights of Plaintiffs, who spent decades helping Shriners Hospitals for Children be successful, Shriners PolicyMakers acted with moral turpitude when placing Plaintiffs under moral duress to pressure them into participating in the administration of investigational drugs.

357. Shriners PolicyMakers intentionally inflicted emotional damage to Plaintiffs by gaslighting them and using them as an example to all other employees that they, too, will be

penalized should they exercise their statutory and constitutional right to refuse the unlicensed, investigational, and federally funded EUA/PREP Act drugs.

358. Such actions are outrageous because Shriners PolicyMakers signed the Provider Agreement, have an FWA, know of the legal requirements under the EUA statute, and spent decades conducting medical research under the same laws regulating investigational drugs in the instant action and promised the federal government, the State, their IRB, and employees that they would not engage in the conduct that is challenged before this court.

359. Shriners PolicyMakers betrayed their oath, promise, loyalty, and commitment to a quality standard of healthcare for personal goals at the expense of Plaintiffs' rights, health, and emotional and financial well-being.

360. The only reason Shriners PolicyMakers deprived Plaintiffs of their Equal Protection and Due Process rights was their refusal to participate in the federal COVID-19 CDC Program in which the State willfully participated.

361. Shriners PolicyMakers violated the terms of the CDC Program when they penalized Plaintiffs.

362. Had the State not agreed to administer the CDC Program through the State's IIS, the investigational drugs would not have been introduced into commerce within the State. Plaintiffs would not have been discriminated against under the EUA Statute and PREP Act.

VII. LEGAL CLAIMS

363. The facts described above constitute a deprivation of several rights guaranteed to Plaintiffs by the United States Constitution, federal statutes, and treaties.

364. These deprivations are actionable under 42 U.S.C. § 1983 because the Defendants acted under color of state law when they deprived Plaintiffs of their right to refuse an EUA/PREP Act product pursuant to the Provider Agreement and the federal statutes cited therein.

365. The *Talevski* case, *supra*, demonstrates that federal statutes and regulations with rights conferring language are enforceable under 42 U.S.C. §1983.[130]

366. The Supremacy Clause doctrine, and the express preemption language in the PREP Act, restrict public and private employers from using state laws that conflict with any provision of the EUA statute and/or the PREP Act, including any at-will employment doctrine.

COUNT ONE

42 U.S.C. § 1983 – Deprivation of the Option to Refuse

367. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

368. The EUA statute and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

369. 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) sets forth Plaintiffs' right to be informed of their option to "accept or refuse administration of the product."

370. Plaintiffs have this right because a drug or device authorized under the EUA statute is not licensed for any legal indication but only authorized for emergency use and has legal distinctions under paragraph 303.

371. Defendants, Shriners PolicyMakers, are an "Organization" under the Provider Agreement and a "vaccination provider" under each EUA letter, acting under color of law, who

[130] *Maine v. Thiboutot*, 448 U.S. 1 (1980), the court held that "Even were the language ambiguous, however, any doubt as to its meaning has been resolved by our several cases suggesting, explicitly or implicitly, that the §1983 remedy broadly encompasses violations of federal statutory as well as constitutional law." See also, *Health and Hospital Corporation of Marion Cty. V. Talevski.*

failed to perform the ministerial act[131] of accepting Plaintiffs' option under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III).

372. Shriners PolicyMakers' refusal to perform the ministerial act is the direct cause of injury to Plaintiffs' financial, emotional, and legal injuries. Such failure stems directly from its actions described above.

373. Ms. Young willfully failed to perform her duties and ensure that Shriners PolicyMakers performed the ministerial act of accepting Plaintiffs' option under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III).

374. The Defendants' actions described above, individually and/or collectively, acting under color of state law, and in deprivation of the Constitutional rights and rights secured by the above federal statutes, regulations, and contracts, unlawfully subjected Plaintiffs to the use of EUA products under threat of penalty outside of their legally effective informed consent as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra.*

COUNT TWO

42 U.S.C. § 1983 – Deprivation of Equal Protection Rights

375. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

376. The Equal Protection Clause of the Fourteenth Amendment to the U.S. Constitution clearly and unambiguously creates rights enforceable pursuant to 42 U.S.C. § 1983.

377. The Equal Protection Clause guarantees equal protection of the laws.

[131] The Ninth Circuit held that officials were only engaged in ministerial acts unprotected from qualified immunity when refusing to allow plaintiffs to apply for a license. *Groten v. California*, 251 F.3d 844 (9th Cir. 2001). Plaintiffs herein were only engaged in ministerial acts unprotected from qualified immunity when refusing to allow Plaintiffs to choose the option to refuse.

378. At all times pertinent, Shriners PolicyMakers, acting under color of law, treated healthcare workers exercising the option to accept differently than healthcare workers exercising the option to refuse, of which both options are under the same section of law (21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)).

379. Shriners PolicyMakers' discriminatory treatment violated the federally funded CDC Program, Provider Agreement, and federal law in deprivation of Plaintiffs' Equal Protection rights under the Fourteenth Amendment.

380. Ms. Young willfully failed to perform her duties and ensure that Shriners PolicyMakers did not deprive Plaintiffs of their Equal Protection rights under the Fourteenth Amendment.

381. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their equal protection rights as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra*.

COUNT THREE

42 U.S.C. § 1983 – Deprivation of Constitutional Procedural Due Process Rights

382. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

383. The Due Process Clause of the Fourteenth Amendment to the U.S. Constitution guarantees the right to due process of law before infringing a citizen's interest in life, liberty, or property.

384. "To have a property interest in a benefit, a person clearly must have more than an abstract need or desire for it. He must have more than a unilateral expectation of it. He must,

instead, have a legitimate claim of entitlement to it." *Board of Regents of State Colleges v. Roth*, 408 U.S. 564 (1972)

385. "Property interests, of course, are not created by the Constitution. Rather, they are created, and their dimensions are defined, by existing rules or understandings that stem from an independent source such as state law -- rules or understandings that secure certain benefits and that support claims of entitlement to those benefits. Thus, the welfare recipients in *Goldberg v. Kelly, supra,* had a claim of entitlement to welfare payments that was grounded in the statute defining eligibility for them." *Board of Regents of State Colleges v. Roth*, 408 U.S. 564 (1972)

386. The right to be free from losing one's finances, education, career, health, dreams, goals, and aspirations in life upon considering whether to participate in the federally funded COVID-19 Vaccination Program is a legitimate property interest claim by Plaintiffs, and such was their expectation when the State agreed to perform for the federal government.

387. The CDC Program, affirmatively adopted by the State, obligated the State to provide the following promised benefits to Plaintiffs: (1) offer the opportunity to participate free of costs, (2) inform individuals of their lawful authority to accept or refuse, (3) provide a Fact Sheet, (4) perform the ministerial act of accepting the individual's chosen option.

388. The right to participate free from outside pressures without costs is a property interest the State promised the federal government it would provide to Plaintiffs.

389. Plaintiffs had an expectation of participating in those benefits.

390. Plaintiffs did participate in those benefits but were deprived of the benefit to refuse without consequence without notice or a hearing.

391. "The fundamental requisite of due process of law is the opportunity to be heard." *Louisville & Nashville R. Co. v. Schmidt*, 177 U. S. 230, 177 U. S. 236.

392. Plaintiffs have the Constitutional right "to present [their] case and have its merits fairly judged." *Logan v. Zimmerman Brush Co*., 455 U.S. 422 (1982).

393. At all times pertinent, Defendants refused to acknowledge Plaintiffs' Constitutional and Statutory rights, thereby nullifying impartiality.

394. Defendants did not provide Plaintiffs with a date, time, place, or procedure to present their case and to have their merits fairly judged by an impartial body before depriving them of their liberty and property.

395. Regardless of any claimed procedure by Defendants, at no time did Defendants acknowledge Plaintiffs' right to refuse the federally funded investigational drugs without consequence.

396. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their procedural due process rights as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra*.

COUNT FOUR

42 U.S.C. § 1983 – Substantive Due Process Rights Regarding Investigational Drugs

397. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

398. The Due Process Clause of the Fourteenth Amendment to the U.S. Constitution guarantees the right to due process of law before infringing a citizen's interest in life, liberty, or property.

399. The EUA statute (21 U.S.C. §360bbb-3) and the implementing statutes and regulations of the CDC COVID-19 Vaccination Program, found at 45 CFR 46, the Belmont

Report, Article VII of the ICCPR Treaty, the Federal Wide Assurance, the EUA Scope of Authorization letters, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

400. The Supreme Court established a two-pronged approach [132] to analyze a substantive-due-process claim.

401. First, the Due Process Clause protects fundamental rights and liberties deeply rooted in this nation's history and tradition. *Moore* v. *East Cleveland,* 431 U. S. 494, (1977).

402. Second, the Court requires a "careful description" of the asserted fundamental liberty interest. *Reno* v. *Flores,* 507 U. S. 292 (1993)

403. Plaintiffs have a fundamental right to refuse an investigational new drug or a drug not approved by the FDA for general commercial marketing according to its labeling.

404. Starting in 1938 (the FDCA Act), the federal government prohibited drugs from being introduced into commerce before the FDA approved them for general marketing according to their labeling (21 U.S.C. § 355(a)).

405. In 1974, Congress passed the National Research Act requiring an institutional review board to monitor research activities involving humans and unapproved therapeutics and devices.

406. In 1978, the Belmont Report was published and formed the basis of the Common Rule (45 C.F.R. Part 46).

407. In 1992, the U.S. Senate ratified Article VII ICCPR Treaty.

[132] *Washington v. Glucksberg*, 521 U.S. 702 (1997)

408. In 2001, the federal government required all persons conducting business with its agencies, departments, or the military to have an FWA on file before they use federal funding when involving humans with investigational medical products.

409. Today, all US States and Territories have an FWA.

410. Most hospitals, county clinics, universities, and medical facilities have an FWA.

411. The State of Texas has more than 1,300 active FWAs alone.

412. Congress established strict requirements upon persons relating to investigational new drugs under 21 U.S.C. 321, 331, 351, 352, 355 360bbb, and 371; 42 U.S.C. 262; 10 U.S.C. § 1107; 45 C.F.R. Part 46; 21 C.F.R. §§ 50,56; 10 U.S.C. § 980; Article VII ICCPR Treaty; 21 C.F.R. § 312; and the Belmont Report.

413. The primary requirement of these statutes, agreements, and treaties is that a person offering a federally funded investigational product must ensure that the potential recipient is never under outside pressure to participate, such as "sanctions," "coercion," "undue influence," and "unjustifiable pressures."

414. The right to refuse an investigational new drug is a fundamental right that is pervasive, historical, and deeply rooted in the fabric of the $600b pharmaceutical research industry and this nation.

415. Moreover, no constitution, statute, regulation, or treaty provides any person any authority to require another person to participate in the administration of an investigational drug under threat of penalty.

416. When the Third Circuit Court of Appeals recently held that "there is no unqualified right to decide whether to 'accept or refuse' an EUA product without consequence,"[133] it ruled in error and upended an entire nation of laws regulating the unlicensed use of drugs.

417. Shriners PolicyMakers, acting under color of law, deprived Plaintiffs of their substantive due process (i.e., fundamental) right to refuse the injection of an investigational new drug without penalty when Shriners penalized Plaintiffs for making their individual choice.

418. Ms. Young willfully failed to perform her duties and ensure that Shriners PolicyMakers did not deprive Plaintiffs of their substantive due process (i.e., fundamental) right to refuse the injection of an investigational new drug without penalty when Shriners penalized Plaintiffs for making their individual choice.

419. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their substantive due process rights to refuse an IND as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra*.

COUNT FIVE

42 U.S.C. § 1983 – Substantive Due Process Rights under the PREP ACT

420. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

421. The Due Process Clause of the Fourteenth Amendment to the U.S. Constitution guarantees the right to due process of law before infringing a citizen's interest in life, liberty, or property.

[133] *Children's Health Defense Inc. v. Rutgers, the State University of New Jersey*, No. 22-2970 (3d Cir. 2024)

422. The PREP Act (42 U.S.C. §§247d-6d,6e) and the implementing statutes and regulations of the CDC COVID-19 Vaccination Program, found at 45 CFR 46, the Belmont Report, Article VII of the ICCPR Treaty, the Federal Wide Assurance, the EUA Scope of Authorization letters, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

423. Individuals have the explicit and fundamental right to due process.

424. There has not been a time in American history when an entire nation was subject to the loss of due process in direct violation of federal law, as witnessed during the last three years.

425. All drugs and devices under any mandate by any person were listed as covered countermeasures under the PREP Act.

426. As described above, the PREP Act provides immunities by depriving participants of their right to seek judicial relief in the event of injury from a PREP Act countermeasure.

427. Relinquishment of that right must be voluntary and not compulsory.

428. The actions of Ms. Young and Shriners PolicyMakers described above violated Plaintiffs' substantive due process rights when Defendants required Plaintiffs' nonconsensual use of covered countermeasures as a condition to continue employment within the State's licensed healthcare industry.

429. The PREP Act expressly preempted such moral turpitude.

430. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their substantive due process rights under the PREP Act as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra*.

COUNT SIX

42 U.S.C. § 1983 - Unconstitutional Conditions Doctrine

431. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

432. The EUA statute (21 U.S.C. 360bbb-3), the PREP Act (42 U.S.C. §§247d-6d,6e), and the implementing statutes and regulations of the CDC COVID-19 Vaccination Program, found at 45 CFR 46, the Belmont Report, Article VII of the ICCPR Treaty, the Federal Wide Assurance, the EUA Scope of Authorization letters, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

433. "[T]he state, having power to deny a privilege altogether, may grant it upon such conditions as it sees fit to impose. But the power of the state in that respect is not unlimited; and one of the limitations is that it may not impose conditions which require the relinquishment of constitutional rights. If the state may compel the surrender of one constitutional right as a condition of its favor, it may, in like manner, compel a surrender of all. It is inconceivable that guaranties embedded in the Constitution of the United States may thus be manipulated out of existence (emphasis added)". *Frost Trucking Co. v. R.R. Com*, 271 U.S. 583, 593-94 (1926)

434. A state-issued medical license is a property interest belonging to the Plaintiffs.

435. Ms. Young established a State-enforced custom acted upon by Shriners PolicyMakers of an unspoken requirement that to enjoy the use of their medical licenses; Plaintiffs had to surrender their liberty interest to refuse the EUA/PREP Act investigational drug.

436. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, established an

unconstitutional condition described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra*.

COUNT SEVEN

Breach of Contract, Third Party Beneficiary

437. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

438. Plaintiffs have enforceable third-party beneficiary rights under the CDC Provider Agreement.

439. The third-party beneficiary right intended for Plaintiffs by the Provider Agreement under the EUA statute is right to be informed of the option to accept or refuse an EUA product without penalty.

440. The third-party beneficiary right intended for Plaintiffs by the Provider Agreement under the PREP Act is right to be informed of the voluntary nature of a PREP Act program.

441. Shriners PolicyMakers breached both of those third-party beneficiary rights of Plaintiffs when Shriners penalized Plaintiffs for refusing an EUA/PREP Act investigational drug.

442. Ms. Young breached both of those third-party beneficiary rights of Plaintiffs when she failed to ensure that Shriners did not penalize Plaintiffs for refusing an EUA/PREP Act investigational drug.

443. The Defendants' actions described above, individually and/or collectively, and in breach of the CDC COVID-19 Vaccination Program Provider Agreement, deprived the Plaintiffs of the benefits intended to be conferred upon them through the terms and conditions of the CDC COVID Vaccination Program Provider Agreement as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra* .

COUNT EIGHT

Wrongful Termination

444. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

445. The Supremacy Clause doctrine, and the express preemption language in the PREP Act preempt State laws and legal requirements that conflict with the EUA statute and/or the PREP Act.

446. As such, Defendants cannot use Texas's at-will employment doctrine to penalize Plaintiffs for exercising their right to refuse an EUA/PREP Act investigational drug.

447. The Plaintiffs did not consent to the deprivation of labor, wages, or employment.

448. The Defendants' actions, individually and/or collectively, and in derogation of Texas's common laws, unlawfully terminated Plaintiffs' employment in violation of the public policy exception and the federal statute's preemption of that at-will employment doctrine as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra* .

COUNT NINE

Intentional Infliction of Emotional Damage

449. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

450. Despite the United States Congress prohibiting Defendants from penalizing potential recipients of EUA/PREP Act drugs who exercised their right to refuse, Defendants engaged in a scorched earth policy and inflicted, with malicious intent, severe emotional distress

to the fullest extent of their positions of authority and power to the detriment of Plaintiffs' emotional well-being.

451. To prove intentional infliction of emotional distress, Plaintiffs must demonstrate that Shriners PolicyMakers (1) acted intentionally or recklessly; (2) engaged in extreme and outrageous conduct; (3) caused Plaintiffs emotional distress; and (4) caused emotional distress that was severe. *Hoffmann--La Roche Inc. v. Zeltwanger*, 144 S.W.3d 438, 447 (Tex. 2004).

452. Defendants knew or should have known, that they could not engage in their reckless conduct nor pressure Plaintiffs to use federally funded COVID-19 EUA/PREP Act drugs by using the tactics of depriving Plaintiffs of their chosen profession and use of their state-issued medical licenses.

453. The facts and the Defendants' conduct committed with gross negligence, reckless, or intent, as described above in the Complaint, give rise to a claim of intentional infliction of emotional distress under the common law of the State of Texas against the Defendants for the damages described in Paragraphs 457 through 462, *infra*.

COUNT TEN

Implied Private Right of Action 21 U.S.C. §360bbb-3

454. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 366, as if fully set forth herein.

455. Should the court not agree that Shriners PolicyMakers were engaged in State Action, Plaintiffs claim that 21 U.S.C. §360bbb-3 contains an implied private right of action pursuant to *Cannon v. University of Chicago*, 441 U.S. 677 (1979), *Wilder v. Virginia Hosp. Ass'n*, 496 U.S. 498 (1990), and *Cort v. Ash*, 422 U.S. 66 (1975).

456. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty have deprived the Plaintiffs of their explicit right to refuse the administration of an emergency use authorized drug and/or medical product without penalty as described in the above facts, thereby causing them damages described in Paragraphs 457 through 462, *infra*.

VIII. DAMAGES RECOVERABLE AND DEMANDED

457. The following paragraphs are hereby incorporated by reference into Counts One through Ten, as if set forth here *in extenso*.

458. As a direct and proximate result of the Defendants' unreasonable and unlawful actions, Plaintiffs have suffered past damages and will suffer future damages, both compensatory and general, including, but not limited to, front and back pay; loss of benefits; loss of accumulated sick pay; loss of retirement accounts; lost earnings on retirement funds; vacation time, compensatory time, and paid time off; negative tax consequences (in the event of a lump sum award), including related accountant fees; attorney's fees; emotional distress; mental, psychological and physical harm; loss of income; loss of enjoyment of life; for which defendants are liable in compensatory, punitive, exemplary, legal, equitable, and all other damages that this Court deems necessary and proper.

459. When the Defendants' behavior reaches a sufficient threshold, punitive damages are recoverable in § 1983 cases. *Smith v. Wade*, 461 U.S. 30 (1983).

460. Because Defendants' actions were intentional, willful, reckless or with callous indifference to the Plaintiffs' federally protected rights, or motivated by evil motive or intent, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each Defendant

in an amount sufficient to deter them, individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

461. Plaintiffs seek recovery of attorney's fees under the Civil Rights Attorney's Fees Awards Act of 1976 and 42 U.S.C. § 1988, and under any other provision of law or basis.

462. Plaintiffs seek recovery of all court costs and out-of-pocket litigation expenses, including but not limited to expert fees, and legal interest on any amount of damages awarded.

IX. JURY TRIAL DEMAND

463. Plaintiffs are entitled to, and hereby demand, a trial by jury on all issues of fact.

WHEREFORE, Plaintiffs pray that Defendants be served with a copy of this Complaint and be duly cited to appear and answer same, and after due proceedings are had, there be judgment herein against the Defendants awarding Plaintiffs all damages claimed herein, plus legal interest, taxable costs, expert fees, and attorney's fees, and all other relief determined to be just and equitable by this Court.

CERTIFICATE OF SERVICE

I hereby certify that on this 20th day of February, 2024, I presented the foregoing pleading to the Clerk of Court for filing and uploading to the CM/ECF system, which will send notification of such filing to all counsel of record.

/s/ David J. Schexnaydre
DAVID J. SCHEXNAYDRE

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